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FCF|REACT Cash & Livelihoods Monitoring and Evaluation

Baseline (+PDM) Report

USAID

GHR FOUNDATION

Save the Children

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This report was developed by:

SPEAR INSIGHTS

April 2021

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This publication was made possible by the generous support of the European Union (EU) and the United States Agency for International Development (USAID). The contents are the responsibility of the lead consultant and Save the Children and do not reflect the views of the EU or USAID.

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1. Executive Summary

This report presents a combination of results and includes both a Baseline study for key indicators, as well as post-distribution monitoring for indicators related to the cash transfer component of the program. The results of the Baseline (+PDM) study are very positive. It is clear that the cash transfers produce significant benefits for the families as 96% of respondents think the cash is the most effective form of assistance for the family to meet their basic needs. The results are also positive in keeping the families together as the evidence points to reduced family separations since the cash transfers began. Yet, with that noted, there were still some family separations.

Currently, only 4% of respondents reported that they can fulfil all of their basic needs while 69% can only fulfil some of them and spending on children's food and education are still major concerns for almost half of all households interviewed. Some children have had to stop their education. The hope and expectation are that continued cash and the addition of livelihoods assistance will continue to reduce or stop further separations. Some families, however, remain in a difficult position.

There are many positive indicators from the project and there are very few reported negatives of the cash. There was, however, one family in which arguments over how to spend the money had begun and domestic violence had increased due to this. Furthermore, there were three reports of issues with respondents claiming to feel at risk collecting the money in the communities and six respondents who had concerns about others in the community being jealous or talking about them receiving the assistance. These concerns would appear not to be widespread and are outweighed by the overall positives of the project. In other results, 82% of women reported shared decision-making on cash transfer use and 96% of the respondents reported that the assistance was delivered safely.

In all cases that children started work the 'household had insufficient income to meet their basic needs' was cited. This is an indication, and further highlights the need for the cash transfers and livelihood developments to stop the separation of children from parents and caregivers, and to keep children out of work.

11% of children who were in basic education before COVID-19 have since stopped. Among them the reasons varied. The most voiced reason for children having to start work is due to the family not being able to afford school (29%). As such, it can be concluded that not having enough money is the biggest causal factor in children not attending or dropping out of school, therefore, the project implementations are on target; to support the families to keep their children in or to send their children to school.

The cash transfers were used by almost every respondent (97%) towards the purchasing of food, secondly towards childcare (79%) and thirdly towards health & medication (59%). This in in line with the respondent's vulnerable positions. Each of these most used expenditures is a basic need of the families.

Information provided to the households regarding the cash transfers has room for improvement. Almost half (44%) of the respondents think that the cash transfers are supposed to last them for two weeks; yet the cash transfers are actually monthly. Furthermore, 54% of the households reported that they received clear information about the assistance they would receive, including the amount and only 40% could identify at least one available channel for providing complaints or feedback about the assistance.

As COVID-19 currently continues to spread in Cambodia, (March 2021) and at faster rates, there are still many concerns for the vulnerable recipients of the assistance. Continuing the cash and livelihood assistance is unequivocally judged as a positive benefit to those involved at the time of this Baseline.

For further improvements to the program some recommendations are listed below. For further details please refer to the recommendations section at the end of this report.

1. Continue to implement the cash transfers to reduce family separations.
2. Improve the communication with recipients on cash delivery.
3. Improve messaging on how long the cash should last and how to budget for their usage.

4. Improve messaging on channels to file complaints or provide feedback.
5. Conduct awareness raising on safe migration practices to help families who must travel.
6. Case workers should include discussions about stress, tension and anxiety reduction.
7. Consider the inclusion of food provisions in addition to cash transfers.
8. Focus livelihood development activities on animal raising, agriculture and small shop/vendor.

2. Context

2.1 The Child-Parent Relationship in Cambodia

Cambodia has made great improvements concerning children's health, education, and their protection, strongly driven by Cambodia's impressive economic growth during 1995 to 2017, when Cambodia was the sixth fastest-growing economy in the world and the poverty rate fell from 47.8% to 13.5%. Labor force participation increased to approximately 84 per cent, around 80 per cent for women and around 89 per cent for men in 2017 (National Institute of Statistics, 2018). As a result, Cambodia's economy has shifted from a low, to middle-to-low, income country, with the ambition of becoming a middle-income country by 2030.

Despite these nationwide accomplishments, a complex array of factors still prevent many children from reaching their full potential. Cambodia's population is still roughly 90% rural, and multidimensional poverty remains high with the expectation of the poverty rising to 17.6% due to the Covid-19 pandemic (The World Bank, 2020; UNDP, 2020).

The engagement of both parents in making an income has posed challenges regarding the care of children by their parents. Children, suggests Murage (2019), do not have much time with their parents because they are busy with their work, and many children do not see their parents until the weekend. Cambodian children aged below 15 years are often left behind with grandparents, hired caregivers or other relatives, instead of being with their working parents (Zimmer and Natta, 2015; Laidler, 2018). The absence of parents in caring for children at home due to their work, or migration for work, can lead children to take on the care-giving roles in the family, instead of their parents. Older children are often responsible for taking care of younger siblings when their parents are occupied with their business (Murage, 2019) and sometimes children are forced to drop out of school to help with the household chores, caring work or other needs to support the family (Laidler, 2018).

In poorer families, parents or caregivers sometimes send their children to Residential Care Institutions (RCIs) to obtain care, food and education for their children (Csáky, 2009). RCI's have been proven to be more costly than family-based care and can have a number of harmful impacts on a child's development. Some RCIs maintain relationships with the children's' parents, but others have been known to prevent a child's contact with their parents. RCI's have been known to send children to other, further locations without their parents' consent (Better Volunteering Better Care, 2014). Furthermore, some RCIs in Cambodia have been involved in child trafficking and child abuse (Farley, 2015; (Murdoch, 2015).

2.2 Policy

Due to the increase in the number of children being placed in RCIs, the Cambodian government issued a Policy Paper on 'Alternative Care for Children in Need of Special Protection' and 'Children at Risk' in 2006 (MoSVY, 2006). Principles in the policy paper include:

- Residential care should be a last resort and a temporary solution for children.
- Family-based care and community-based care are the best choices for alternative care.
- Specific strategies and measures shall be developed to support parents to raise their children and send them to school.

- These strategies and measures shall also be directed to families, relatives and communities caring for children where parental care is not possible, to avoid institutionalization.
- Where residential care cannot be avoided, minimum standards and guidelines for residential care are to be implemented, monitored and evaluated in accordance with the overriding principles of the 'Convention on the Rights of the Child.'
- Residential care and/or community or family-based care are requested to strive to meet minimum standards developed within the framework of this Policy. NGOs which provide alternative care service must request permission from and sign an agreement with the Ministry of Social Affair, Veterans and Youth Rehabilitation (MoSVY).

Besides, the Sub-decree on the Management of Residential Care Institutions was developed and came into effect in September 2015 intending to put in place 'gatekeeping' mechanisms to reduce the intake of children into residential institutions (MoSVY, 2015).

2.3 Program Background

FCF|REACT is a network of organizations working together to support children to live in safe, nurturing, family-based care facilitated by Save the Children (SCI). SCI works in collaboration with the government, local and international NGOs, academic institutions and UN agencies, to enhance and strengthen family-based care. SCI, together with around 60 member organizations, are working to prevent children from being separated from their families and increase the number of children who are safely and successfully integrated into family-based care. The intervention is done to strengthen systems and policies and to provide services to children and families directly.

The COVID-19 pandemic threatens children and their rights and exposes them to potentially significant disruption to their healthcare, education, access to basic needs and services such as food, protection and social interaction. Loss of income and inflated prices will increase food insecurity among the most vulnerable families, intensifying the risks of family separation as poverty is recognized as a significant factor in the placement of children in RCIs.

(A family receiving support during COVID-19 pandemic, CCT 2020)

By implementing child-sensitive livelihoods to secure/restore basic incomes and providing child-sensitive cash transfer assistance, SCI aim to enable vulnerable families to access sufficient resources to invest adequately in their children, to help children and families to stay together safely, and mitigate the economic impact of COVID-19 in the medium term.

3. Objectives

3.1 Objectives of FCF|REACT Cash & Livelihoods

The FCF|REACT 'Cash & Livelihoods' program, with support from USAID, was created with the aim to improve the capacities of vulnerable families affected by the secondary impacts of COVID-19, to stay together in a caring and nurturing environment and to reduce protection risks for their children through two components:

- Child Sensitive Cash transfers assistance to support households in situations of high economic insecurity due to the impacts of COVID-19 to prevent them from resorting to negative coping strategies that could lead to increased child protection risks.
- Child sensitive livelihood promotion interventions, including training and micro-grants provision for vulnerable households and youth who have lost their sources of income due to COVID-19, to prompt livelihoods restoration and prevent the use of negative coping strategies potentially harmful for children, in a sustainable manner.

(A family getting introduced and oriented about the cash transfer program, CCT 2020)

3.2 Objectives of the Baseline (+PDM) study

The objectives of the Baseline (+PDM) study are to:

- Establish the Baseline values for the FCF|REACT Cash & Livelihoods program for the project indicators as identified in the MEAL framework.

- Inform the confirmation or adjustments of indicators and targets under the FCF|REACT Cash & Livelihoods program.
- Undertake post-distribution monitoring for the cash component of the program to understand how the cash transfers are being used by households, the outcomes it is having in households, and to identify any unintended safety and equity issues in the distribution.
- Generate strategic evidence to contribute to the FCF|REACT Learning Agenda

4. Methodology

4.1 Methods and Sample Size

The project Baseline (+PDM) relies on a primarily quantitative framework with fieldwork undertaken with around 10% of project beneficiaries (N=70). Due to the sensitive nature of the project, the quantitative interviews were conducted face-to-face with the respondents in each location, following COVID-19 safe guidelines. Local enumerators were hired in each province and centrally trained in Phnom Penh prior to undertaking the fieldwork.

Due to budgetary constraints, fieldwork is prioritized within the four major locations of beneficiaries including Phnom Penh, Kandal, Siem Reap and Battambang. 34 households from Battambang, 32 from Siem Reap, 2 from Phnom Penh and 2 from Kandal were included in this Baseline study.

To ensure comparability of data, respondents were selected based on their 'score' in the previously conducted Household Economic Survey (HES), which was used to identify households (among those already receiving case management services from partners) that met a scoring threshold to be considered 'eligible' for cash transfers. This score is a calculation of vulnerability based on household composition and socio-demographic factors, economic vulnerability, food insecurity and child protection risks.

The sample below was selected before fieldwork started. Although efforts were made to create the pools as homogeneously as possible, there are some differences among key criteria between the waves of data collection. The column for the Baseline data in the table below is the actual result. For the other stages of fieldwork, it is the planned demographics of the respondents to be selected. Therefore, the actual demographics of the respondents interviewed in each wave may change. Differences between the demographics of the respondents among each wave will be incorporated into the analysis and noted in each report.

	Baseline (+PDM)		PDM1		PDM2		Endline		All Eligible HH	
TOTAL HES Score										
30-39	14	20%	14	20%	14	20%	14	20%	130	20%
40-49	19	27%	20	29%	20	29%	20	29%	189	29%
50-59	24	34%	19	27%	19	27%	19	27%	170	26%
60-69	8	11%	11	16%	11	16%	11	16%	111	17%
70+	5	7%	6	9%	6	9%	6	9%	59	9%
Dependency Ratio										
Less than 1.5	36	51%	26	37%	31	44%	30	43%	293	45%
Dependency Ratio (1.5 to 2)	20	29%	26	37%	19	27%	21	30%	202	31%
Dependency Ratio (2.1 to 3)	8	11%	12	17%	12	17%	10	14%	91	14%
Dependency Ratio (more than 3)	6	9%	6	9%	8	11%	9	13%	72	11%

	Baseline (+PDM)		PDM1		PDM2		Endline		All Eligible HH	
HH w/ Disabled/Chronically Ill										
N/A	20	29%	15	21%	32	46%	27	39%	222	34%
1 person	25	36%	26	37%	20	29%	21	30%	209	32%
More than 1 person	25	36%	29	41%	18	26%	22	31%	222	34%
ID Poor Status										
No	17	24%	19	27%	29	41%	28	40%	227	35%
Type 1	20	29%	23	33%	18	26%	26	37%	219	34%
Type 2	33	47%	28	40%	23	33%	16	23%	206	32%
Indebtedness										
None	16	23%	14	20%	9	13%	29	41%	163	25%
<USD500	20	29%	29	41%	27	39%	20	29%	235	36%
USD500+	34	49%	27	39%	34	49%	21	30%	254	39%
Extent of Income Decrease										
Partly	17	24%	16	23%	20	29%	11	16%	153	23%
Majority	37	53%	32	46%	31	44%	38	54%	325	50%
All	10	14%	14	20%	13	19%	14	20%	122	19%
None	6	9%	8	11%	6	9%	7	10%	52	8%
Household Hunger Scale										
Little to no Hunger	35	50%	26	37%	25	36%	32	46%	306	47%
Moderate	21	30%	30	43%	21	30%	16	23%	183	28%
Severe	14	20%	14	20%	24	34%	22	31%	156	24%
rCSI Score										
<4	3	4%	1	1%	1	1%	1	1%	13	2%
4-19	11	16%	14	20%	15	21%	13	19%	111	17%
19+	56	80%	55	79%	54	77%	56	80%	528	81%
Adopted Harmful Coping Strategies										
No	14	20%	19	27%	16	23%	14	20%	137	21%
One	27	39%	25	36%	21	30%	26	37%	241	37%
More than one	29	41%	26	37%	33	47%	30	43%	274	42%
Likelihood of Separation										
Unlikely	29	41%	24	34%	14	20%	24	34%	196	30%
Likely	17	24%	20	29%	31	44%	24	34%	215	33%
Highly Likely	6	9%	9	13%	9	13%	6	9%	85	13%
Happened	18	26%	17	24%	16	23%	16	23%	163	25%

Green indicates a percentage that is at least 5% higher than the average from all eligible households.

Orange indicates a percentage that is at least 5% lower than the average from all eligible households.

4.2 Baseline (+PDM) Data Collection

To undertake the quantitative data collection, the following steps were taken:

1. **Development of research tools**

The research tools used in this project were created by SCI and utilized a number of existing tools to gather information on the various project indicators. The tools were submitted to the Save the Children US Ethics Review Committee for approval (SCUS-ERC-FY2021-25).

2. Questionnaire translation by the Spear Insights team.

The survey was translated by one member of staff before being edited by a second and a third member. After the third edition, the team met to finalize the translation.

3. Confirmation of the questionnaire by SCI.

Upon completion of the final draft by Spear Insights the translations were sent to SCI for their input or possible suggestions, in particular regarding the correct use of technical terms, before being returned to Spear Insights to finalize, for programming.

4. Programming of the questionnaire into the KoBoToolbox survey tool.

KoBoToolbox was used as it was simple to use for enumerators and allowed data collection through offline mobile devices while in the provinces. The questionnaire was programmed in both English and Khmer as, although it was administered in Khmer, some testing of the programming and analysis of the final results were completed in English.

5. Testing and internal pilot tests of the software.

Usability of the survey, skip logic and typos were tested during internal testing of the software.

6. Confirmation of the data collection software by SCI.

An online link to the survey was shared with SCI to test and confirm the software programming.

7. Two-day fieldwork training

Training occurred over one day and a half to ensure that each enumerator had a clear understanding of the project's objectives, respondent sampling, the format and flow of the questionnaire, ethical concerns and expectations, quality assurance and data monitoring. The training took place under the following guidelines.

a. Morning Day 1: Background and overview of the 'Cash Transfers and Livelihood' program, explanation of the sampling methods and ethical concerns for this project. A first guided read-through of the questionnaire was conducted.

b. Afternoon Day 1: Additional read-throughs of the questionnaire. Q&A with the enumerators. Re-explanation of ethical concerns for this project, interview testing with each of the enumerators and comments from the SCI team.

c. Morning Day 2: Role playing of the survey using the KoBoToolbox software and a final briefing.

8. Data collection was undertaken by the field teams in each province.

The fieldwork team consisted of four enumerators; one in Siem Reap, two in Battambang and one in Phnom Penh. Data collection was undertaken 12-19 March 2021.

9. Data monitoring

During the completion of fieldwork, enumerators called the fieldwork manager in Phnom Penh to check in and discuss any issues, challenges or concerns they had found regarding the fieldwork. These issues included dealing with the unpredictable COVID-19 outbreaks, uncontactable/unresponsive respondents, changes in sampling or suggestions to re-order samples due to uncontactable samples. The team created a telegram group including both the fieldwork team and the team at the head office in Phnom Penh to ensure immediate support for any issues. In addition, the team at the head office in Phnom Penh monitored the data daily, checking key indicators such as interview length, logical responses and the quality of open answers.

10. Analysis of results upon completion of fieldwork.

Data was exported into excel from KoBoToolbox and then imported into SPSS for cross-tabulation of response criteria and data analysis.

(Training on cash transfer tools to partners in Siem Reap, FCF|REACT 2020)

4.3 Data Analysis and Reporting Plans

After completion of data collection, the quantitative data was exported from KoBoToolbox into excel, formatted and then imported into SPSS for analysis. In SPSS, the responses to questions and indicators were cross-tabulated against key respondent identifiers such as gender, disability, IDPoor status level, HHS, rSCI score and likelihood of separation. Usual demographic measurements such as location are unlikely to be used as the project is being analyzed at a project level with a representative sample and as such, there are low numbers of respondents in some locations. The project indicators to be analyzed in each wave of the project are outlined in section '5.0 Evaluation Framework' of this inception report according to the indicator definitions/analysis provided by SCI in the project MEAL Plan.

In subsequent reports after the Baseline (+PDM), changes in results between the waves of the study will be noted. As mentioned in the sample size section earlier, so that the results are comparable, the sample has been selected so that there are minimal differences in the make-up of respondents between waves of fieldwork. Significance of results may be difficult to achieve as the sample sizes used in this research are small. With that noted, significance tests will be applied where possible and any significance in changes will be remarked upon if found.

Normal processes would aim to conduct Baseline, Midline and Endline research on a standard timeline with Baseline before project interventions begin, Midline at the center point and Endline once the project interventions have finished. In the case of this project the Baseline was carried out 3 months after the actual start-up of work. This may have produced some bias in the results due to the limited recalling period and some of the services having already been provided. Furthermore, due to the late start, the gap in time between the data collection phases varies. The time between the Baseline (+PDM) and Midline indicators (which are collected with PDM2) will be 12 weeks (from 19 March to 11 June) but the Endline indicators will be completed approximately six months later at the end of the project. As such, the data is not being analyzed on a consistent timeline which may lead to greater differences in changes between the studies.

In addition, many of the sampled households are also IDPoor and as such receive additional supports compared to non-IDPoor households (although some analysis will be made to compare those within and outside of the IDPoor scheme).

4.4 Ethical Considerations

(A partner staff practicing the household economic survey with a beneficiary, FCF|REACT, 2020)

Before undertaking data collection each of the project partners reached out to the selected respondents and asked for their informed consent to participate in the process and have their contact details shared with Spear Insights. Once the agreement had been confirmed, the Spear Insights team called each of the respondents to schedule their interviews so that they agreed and were aware of the time to expect the enumerators, before their arrival.

Once in the field, the enumerators asked for consent from the respondent within the introduction and the opening question of the survey. The informed consent form contains three main elements including the purpose of the evaluation/survey, the rights to voluntarily participate and/or withdraw from participating and the confidentiality of answers, with the exception of if harm to a child is identified. The consent form was read out loud by the data collectors to the participants to ensure they understood what was written in the form before they voluntarily agreed or not to be interviewed.

All evaluation team members had adequate experience in collecting sensitive information, specifically regarding information concerning violence against women and children. Moreover, the Child Safeguarding policy was introduced to all consultants/enumerators and everyone had to sign the Child Safeguarding policy to ensure that they understood their responsibilities under the policy to protect children from any harm during the evaluation process.

The interviews were conducted in open spaces in response to the respondent's convenience and COVID-19 safety as well. Moreover, consideration of the environment, ensuring that it was free from any disturbance by family members or neighbors was considered by the enumerators, to ensure the confidentiality of the respondents. Financial incentives were not provided to respondents within the evaluation process.

In the event that the enumerators became aware of any harmful situation occurring or becoming apparent during the fieldwork, the process that was put in place was that they would report to Spear Insights' management immediately who would immediately inform SCI for the relevant referral or intervention. If further assistance was needed by the respondents, in terms of social or psychosocial support, the evaluation team would provide the information to SCI, who in turn would inform the partner/caseworker responsible for the household in question, to provide further support. However, during the Baseline (+PDM) data collection, there was no situation identified which required reporting.

The research tools were submitted to the Save the Children US Ethics Review Committee for approval (SCUS-ERC-FY2021-25).

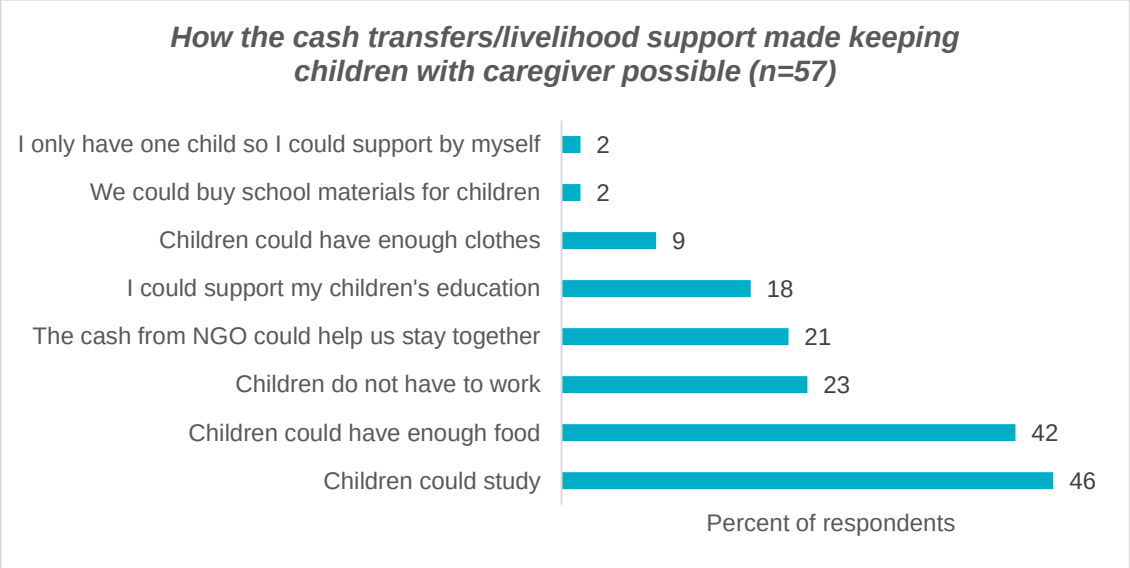
5. Findings

5.1. Percentage of households reporting child separation from a caregiver (including due to work-related migration) because of lack of money.

Key indicator insight:

This indicator refers specifically to family separation (including because of work-related migration) that occurs for the purpose of reducing pressure on household finances or to generate income. If a household reported either children leaving the household because they could not afford to take care of them OR parents/caregivers migrating due to household having insufficient income/lack of money, they will be counted towards this indicator. Overall, 21% (15/70) of households reported child separation from caregiver because of lack of money ***before*** *the first cash transfer distribution by the program in December 2020. 14% (10/70) of households reported child separation from caregiver because of lack of money* ***since the start*** *of the cash transfer distribution.*

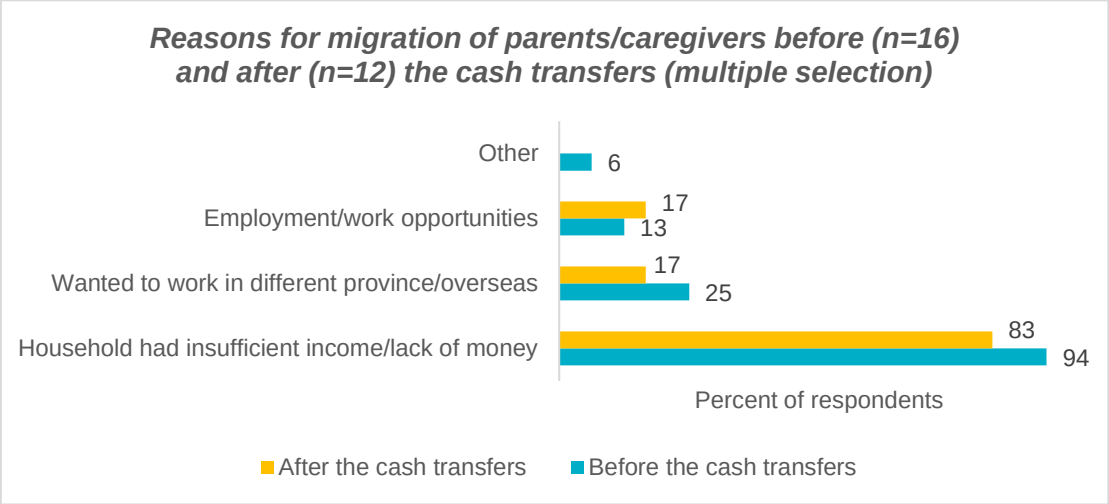
In total, 86% (60/70) of the respondents had the same number of children living with them now as before the cash transfers started, with the remaining 14% (10/70) split with 7% (5/70) having more children and 7% (5/70) having less. Of the 86% (60/70) who remain with the same number of children, 95% of them (57/60) agree that the cash transfers/livelihoods support has made it possible for them to keep all of their children with them. This was mainly because it enabled the children to be able to study (46% - 26/57) to have enough food (42% - 24/57) or to not have to work (23% - 13/57).



The reasons for the families having more children living with them now were mixed with one who came to access healthcare, one who came to access education, one who came for both access to healthcare and education, one for the safety of the child as the parents do not have time to care for the child, and one because the child's parents who have passed away.

Of the 7% (5/70) of households who have less children living with them since before the cash transfers began, none of these separations were cited as being due to insufficient funds, as per the indicator definition. Two of the five respondents with fewer children in the household explained that their children had left home for work, in one household a child had left for work and married, in another the child had married and left to get an education and in the remaining household the child left to access healthcare. The youngest of these five respondents was 36 and each had at least one child over the age of 18 so it is plausible that it was their adult children who left home rather than a young child. This is a limitation of the study and in subsequent data collection rounds reporting of "children" will be limited to those under 18 years old.

23% (16/70) of interviewed households had parents/caregivers of children who migrated before the cash transfers started between March and December 2020, 94% (15/16) of these migrated due to insufficient income or a lack of money. Since the cash transfers began in December 2020, 17% (12/70) of households reported any parents or caregivers migrating. The main reason for this remained that the family had insufficient income, with 83% (10/12) of having a parent/caregiver migrate for this reason. In total, 18 households had a parent/caregiver migrate either before or since the cash transfers. Since the cash transfers started, 39% (7/18) of these households reported that the migrant had returned to the household. However, the cash transfer was not cited as being the reason that made it possible for their return.



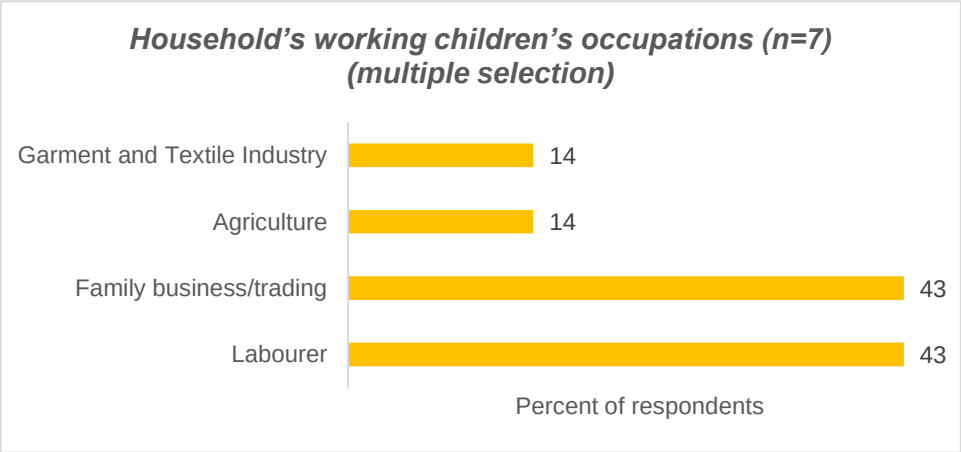
In summary, households have reported few children leaving the household and that the cash transfer has made it possible to keep their children living with them. In addition, fewer of the households have had parents or caregivers migrate since the cash transfers began and some of those who have left, have since returned. Those who returned did not attribute the cash transfers as the reason for them returning, however, as fewer households cited that they had to send a parent/caregiver away due to insufficient income/lack of money it can be surmised that the cash transfers are likely playing a role in helping households staying together.

5.2. Percentage of households that reported having had children go to work during the past 30 days either regularly (on a weekly basis) or frequently (more than once a week), to meet basic needs

Key indicator insight:

Child labor can often be a negative coping strategy used by a household to meet basic needs. If a household reported children going to work in the past 30 days either regularly or frequently because the household had insufficient income to meet basic needs, they will be counted towards this indicator. 10% (7/70) of households reported having had children go to work in order to meet basic needs.

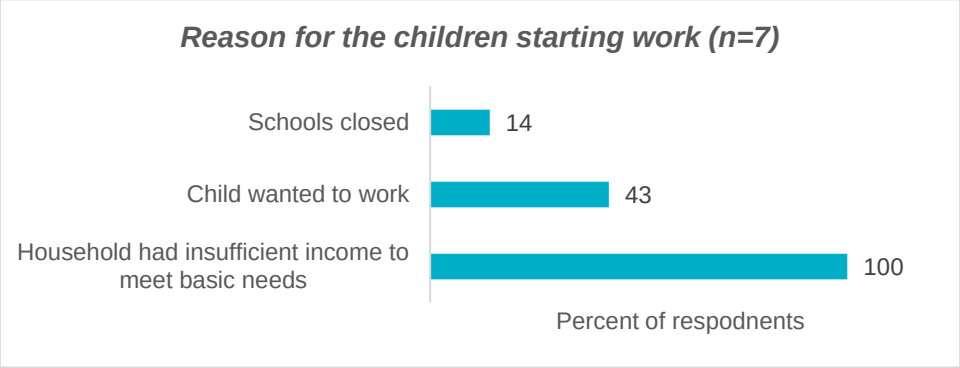
There were 8 children from 7 households who were engaged in work, the youngest children working were 9 and 11 and the average age of children under 18 working was 14. Of the households with children who worked, 3 each indicated that they had children who worked as laborer's and within the 'family business/trading.' None of the working children had a disability but 88% (7/8) were female. The data is not sufficient to indicate a preference of gender to achieve education vs. to be sent to work as in 86% (6/7) of the families it was the oldest child who was working. However, in one case, the family with 2 working children (aged 9 and 11), both were female yet there was an older male aged 13 who was in education and not working.



Of the 7 households who had children working at the time of the survey, 86% (6/7) said that their children worked regularly (i.e., in recurring shifts) and 71% (5/7) worked more than once per week. Of the 5 who worked more than once per week, 3 worked 6-10 hours per week, 1 of them 21-30 hours per week and the other 1 worked more than 30+ hours per week.

However, there is a chance that if the work is not substantially time intensive or perhaps involves helping the family, the parents answering the survey might not consider it as working and thus there may be more children who are informally working but not accounted for within this data.

Among all 7 households that had children working, the household having insufficient income to meet basic needs was at least one of the reasons as to why the children had started working. Among 43% (3/7) of the respondents, they also claimed that 'the child wanted to work' and one parent also explained that their reason was because the 'schools are closed.' The fact that all parents with children working were doing so, at least in part, to support the household income further highlights the need for the cash transfers and livelihood support to prevent child labor.

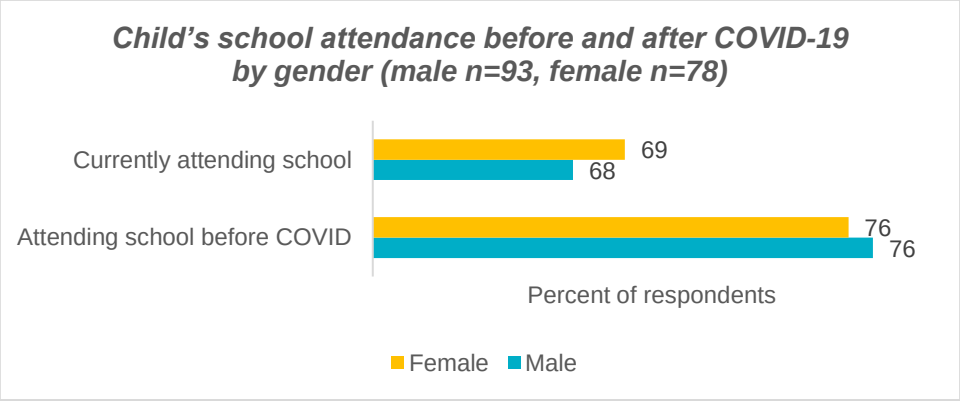


5.3. Percentage of children who were accessing basic education before COVID and are now no longer doing so

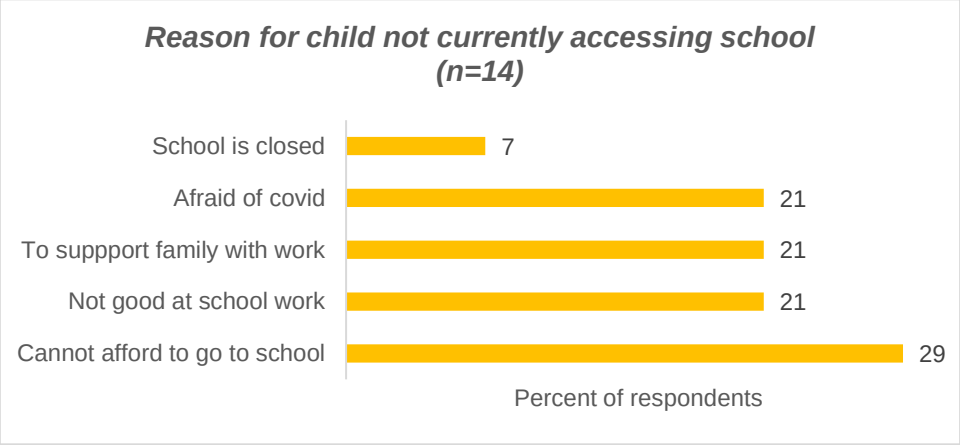
Key indicator insight:

If a child was reported as accessing basic education (primary school and lower secondary) before COVID-19 and is currently reported as not accessing basic education either in-person or remotely, they will be counted towards this indicator. Overall, 11% (13/118) of children who were accessing basic education before COVID are no longer doing so.

Before COVID-19 76% of children (130/171) were studying in school, 118 of them in primary or lower secondary education. Since the pandemic, 8% (14/171) of all children have stopped studying, all but one of which was in basic education and counted towards the indicator, although 2% (3/171) have started attending school who were not before COVID-19. From the sample size there was no discernible difference between male and female withdrawal from school and all 3 children with a disability who were attending school before COVID-19 were still attending school at the time of the study.



For the children who stopped attending school the primary reason was due to not being able to afford school (29% - 4/14). With 21% (3/14) of respondents each, the next reasons were being afraid of COVID-19, the child not being good at schoolwork and to support the family with work. As we know from the earlier questions, the most often cited reason for children having to join in work is due to the family not having enough income. As such, it can be concluded that not having enough money is by far the biggest causal factor in children not attending or dropping out of school. As such, the project implementations are on target to support the families to keep their children in or to send their children to school.

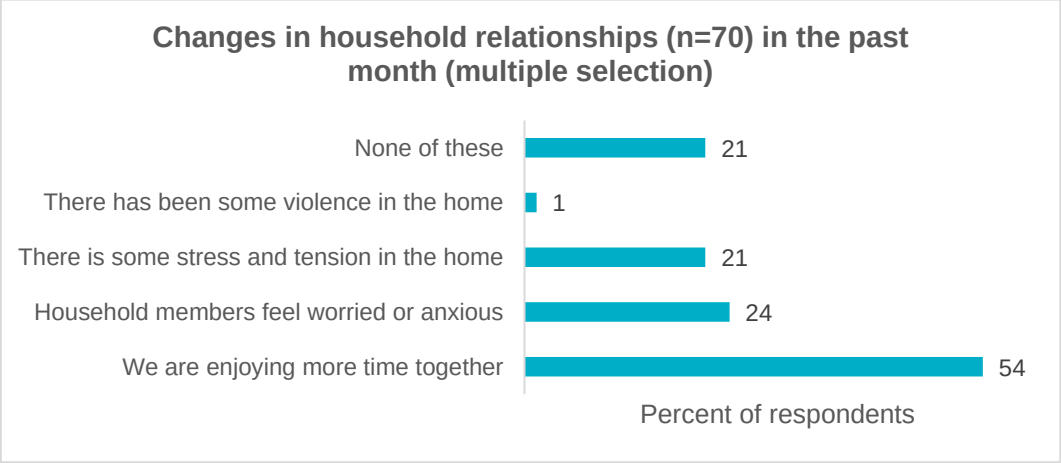


5.4. Percentage of households that report feelings of stress and anxiety

Key indicator insight:

The impact of COVID-19, including income loss, can lead to additional stress and anxiety within households. This stress and anxiety not only affect the psychosocial wellbeing and mental health of household members, but it can also be a driver of violence against children. This indicator intends to give an indication of reporting feelings of stress and anxiety within a household. Overall, 36% (25/70) of households reported that in the past month there has been more stress and tension in the home and/or household members feel worked or anxious and/or there has been some violence in the home.

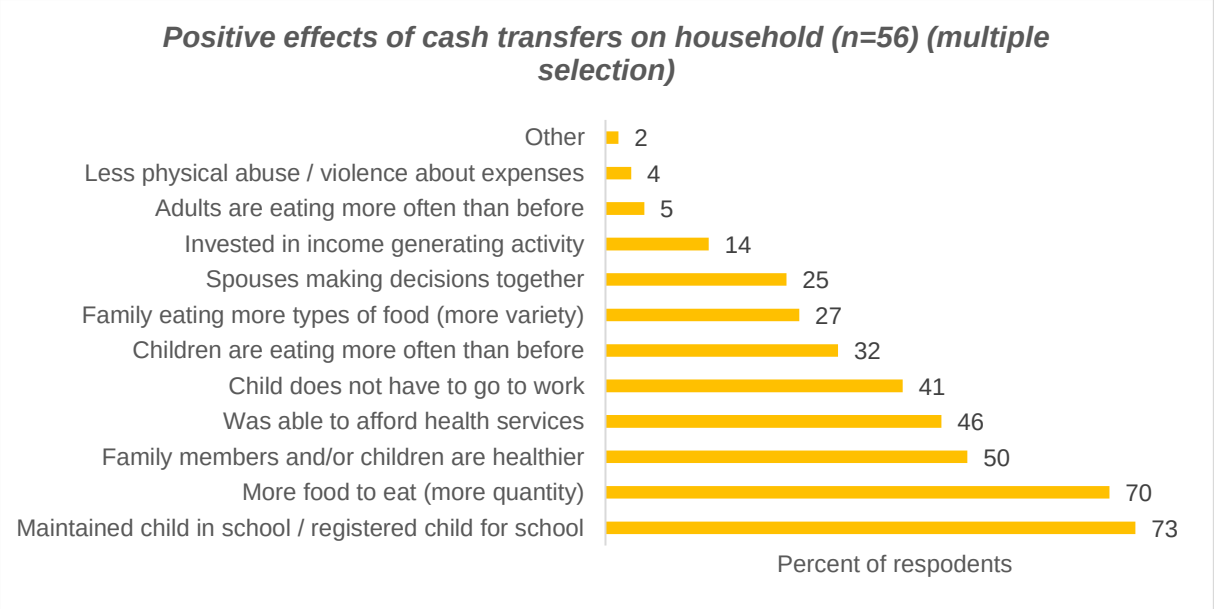
54% (38/70) of respondents reported that their households have been able to enjoy more time together in the past month, however, 24% (17/70) have reported feeling more worried or anxious and 21% (15/70) have reported there is more stress and tension within the home. One respondent reported that there had been some violence in the home and that it had been increasing. This person described that it had been due to disputes with their spouse. In question 22 which asked about 'any negative effects on your household or relationships with your household' this same respondent was the only one to mention negative effects. They claimed that the cash transfers caused "verbal abuse about how to spend the cash."



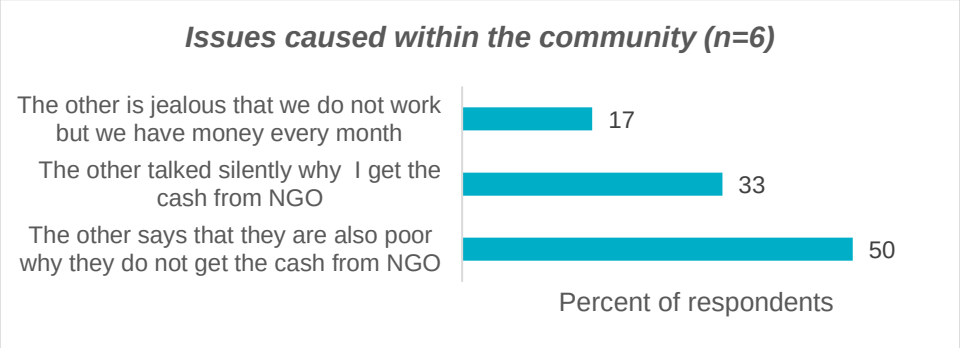
5.5 Positive and Negative Effects of Cash Transfers on Households

Key indicator insight:

In total, 80% (56/70) of the respondents reported that the cash transfers had a positive effect on their household or relationship within them. 73% (41/56) of those whose cash transfer had a positive effect noted that the cash had maintained their child in school/registered their child for school. 70% (39/56) claimed they now have more food to eat and 50% (28/56) said that their family members and/or children are healthier among other reasons. Importantly, 41% (23/56) of respondents noted that their child not having to go to work is a positive effect of receiving the cash transfer. Only 1 person went on to report that there had been any negative effects on their household or relationships within them, this was the same person as mentioned above regarding violence within the household. They commented that there was 'verbal abuse about how to spend the cash.'

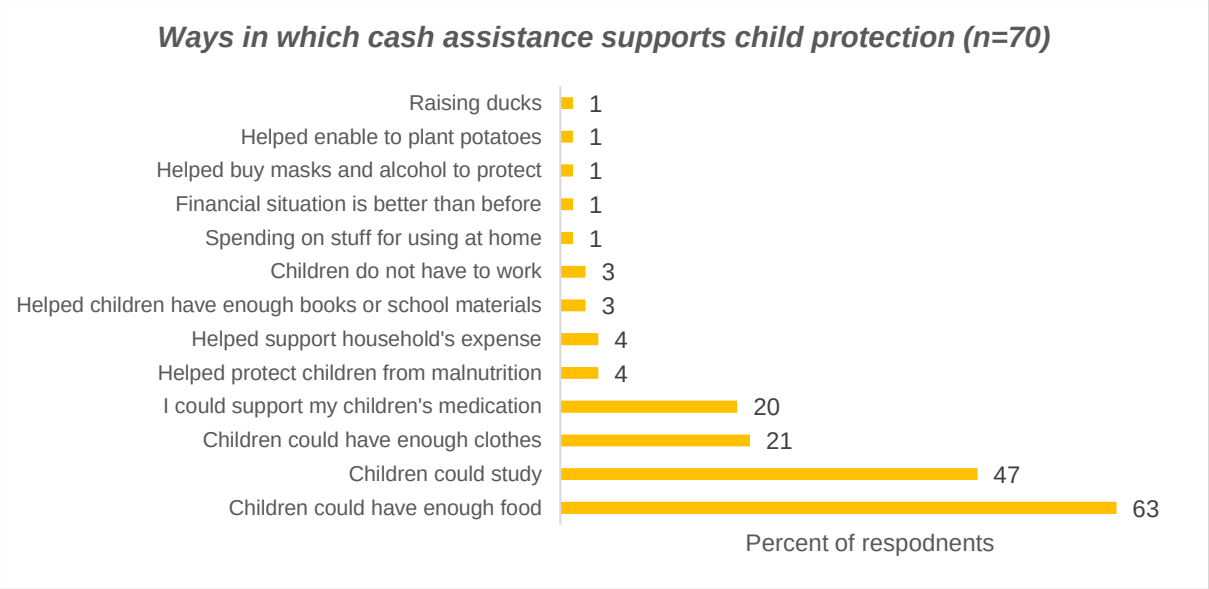


9% (6/70) respondents noted that the cash transfers had caused issues within their communities, all three reasons noted are linked to a feeling of missing out/jealousy of other members of the community for not receiving the same assistance.



All 70 households interviewed said that receiving cash assistance has helped address child protection concerns for the children in their care. The respondents were asked how it had helped address their concerns in an open question, and so reflects how the respondent defined 'child protection concerns.' The assistance was reported to have helped in a number of ways, most frequently to ensure that the children could have enough food (63% - 44/70), could study (47% - 33/70) have enough clothes (21%

-15/70), or have medication (20% - 14/70). Although in section 5.1 above 41% (23/56) of respondents noted children not having to go to work as a positive effect of receiving the cash transfer, in this open question on helping address child protection concerns, only 3% (2/70) of respondents reported that the cash transfer assistance meant that their children did not have to go to work.



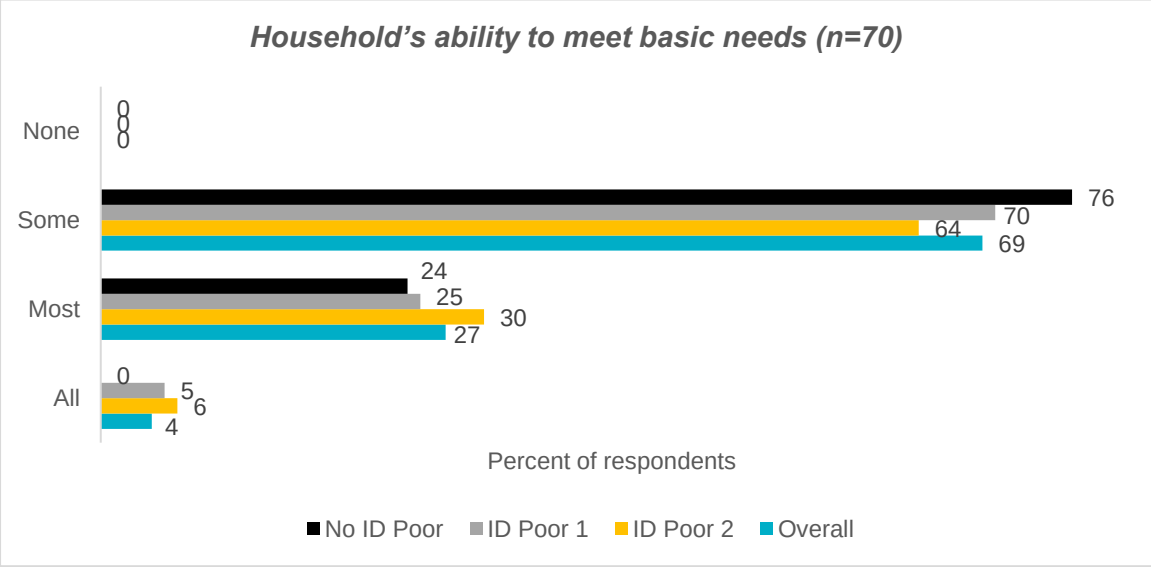
5.6. Percentage of households who report being able to meet the basic needs of their households (all/most/some/none) according to their priorities

(A family eligible for the cash transfer assistance, CCT 2020)

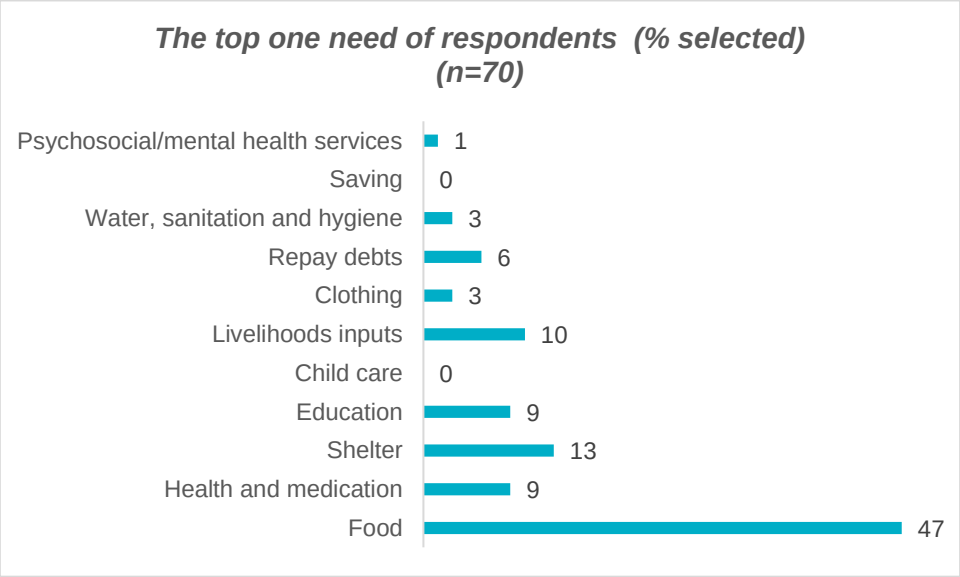
Key indicator insight:

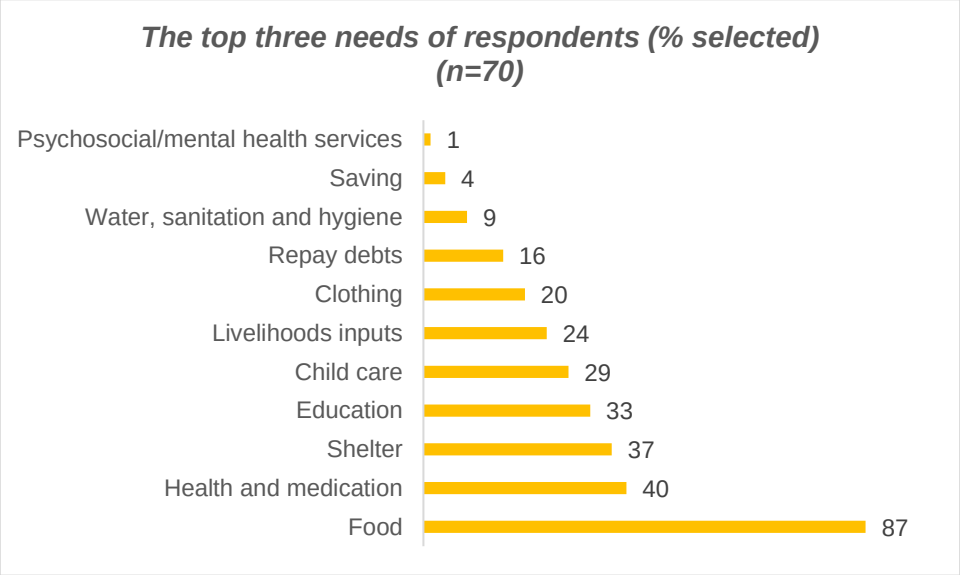
The concept of basic needs refers to the essential goods, utilities, services or resources required on a regular or seasonal basis by households for ensuring long term survival AND minimum living standards, without resorting to negative coping mechanisms or compromising their health, dignity and essential livelihood assets. This indicator attempts to measure how well beneficiary households are able to meet their basic needs (as they define the term).

Amidst all households, 4% (3/70) of them claimed that they can fulfil all their basic needs, followed by 27% (19/70) who can fulfil most of their basic needs whilst most (69% - 48/70) can still only fulfil some of them. None of the respondents claimed that they were unable to fulfil any of their basic needs which is positive. 96% (67/70) of the respondents cited the cash assistance as being the most effective method of helping them and their families to meet their basic needs. Across IDPoor levels there was no significant difference in the results.

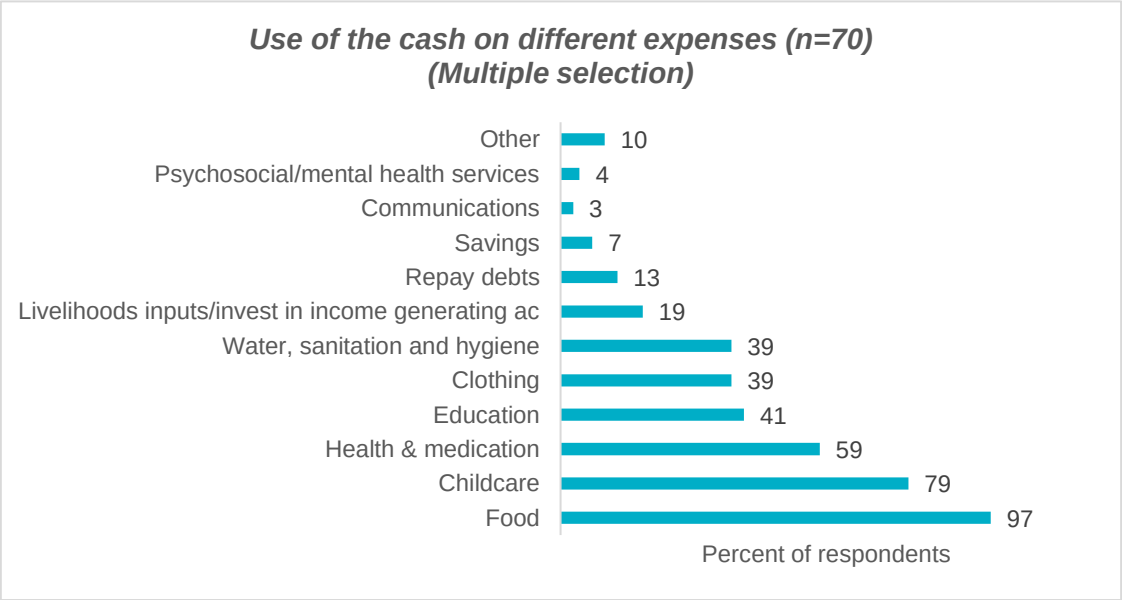


Each respondent was asked to rank their top three priority needs of their household. Food was the most important item for 47% (33/70) of respondents. In total, food was mentioned as a top three priority for 87% (61/70) of households, more than twice as often as the next item 'health and medication' with 40% (28/70).





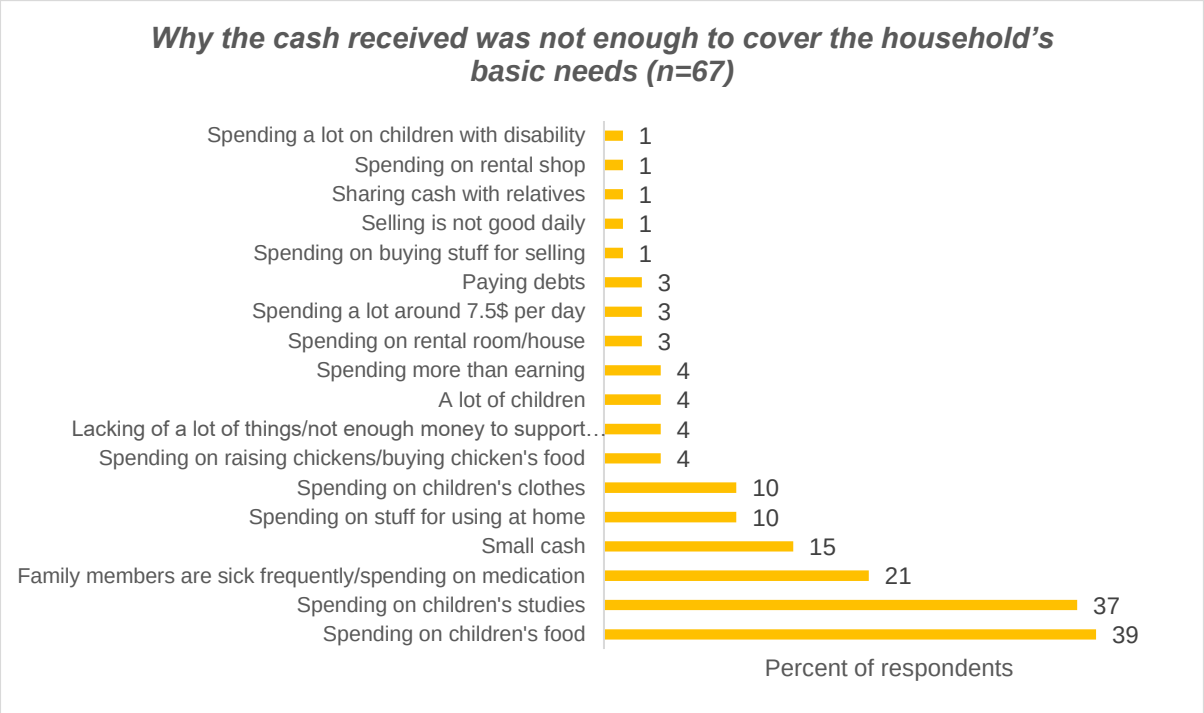
When asked what type of expenses the last cash transfer was used for (multiple selection), almost every respondent (97% - 68/70) reported using it towards the purchasing of food, secondly towards childcare (79% - 55/70) and thirdly towards health & medication (59% - 41/70). The result of this question is partially aligned with the respondent's ranking of what their priority needs of their household are. Before and after the first cash transfer the beneficiaries were involved in economic prioritization sensitization sessions which will likely have influenced their priorities and spending habits. Two key differences between their priorities and their reported spending are that (1) childcare is much lower down on the reported priority needs of respondents compared to the number of people who have spent their cash towards it and (2) shelter is the second most needed priority, yet it is not mentioned within the cash usage. With that noted, the chart above is only the percentage of respondents who have spent money towards each item, not how much they have spent on the item; therefore, it doesn't paint a full picture of the respondents needs. For instance, it may be that most families have spent money on childcare, but the amount of money spent is very low.



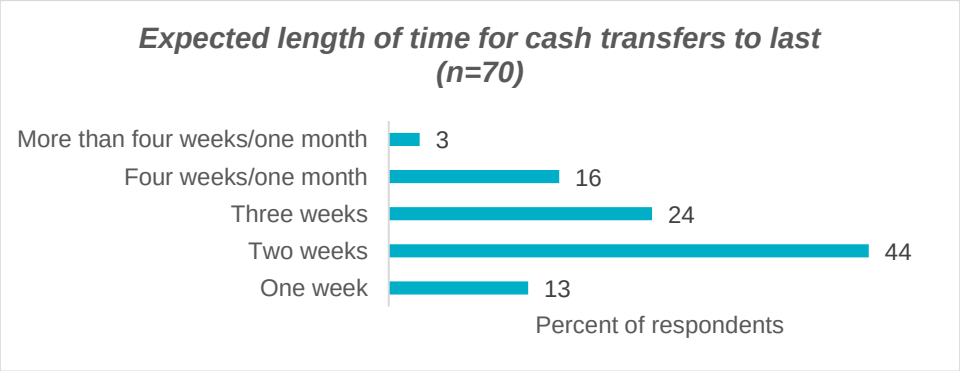
Of those who mentioned that the money was used for 'livelihoods inputs/invest in income generating', all 13 reported that the cash received was helpful to restart existing/develop new livelihood activity. The money was helpful for a variety of reasons, including using as capital, for construction and buying

chickens/ducks, although most of the responses to this question did not identify a livelihood input/investment.

Of the 96% (67/70) of respondents who did not report that their household was able to meet all its basic needs in the last month. Although the cash transfers are not expected to cover the entirety of the households needs, the question as to why the households were not able to meet their needs was interpreted as referring to the cash transfers. The households typically mentioned that their expenditures on their various needs exceeded that which the cash transfers provided them with. 39% (26/67) of respondents identified that spending on their children's food was a reason their cash transfer was not enough to cover their basic needs and 37% (25/67) claimed that it was spending on children's education that meant they did not have enough left for basic needs.

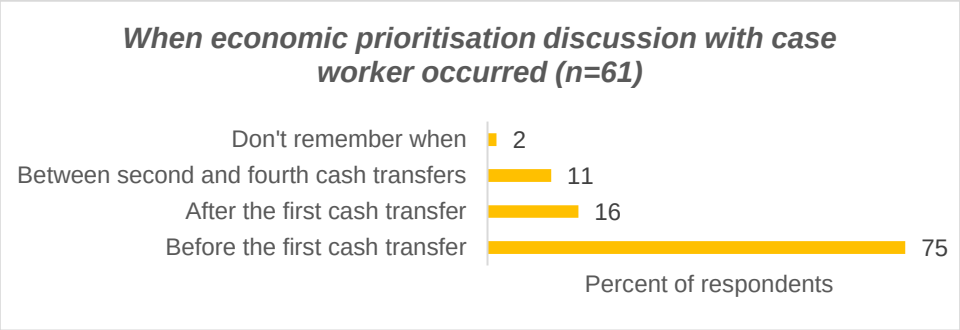


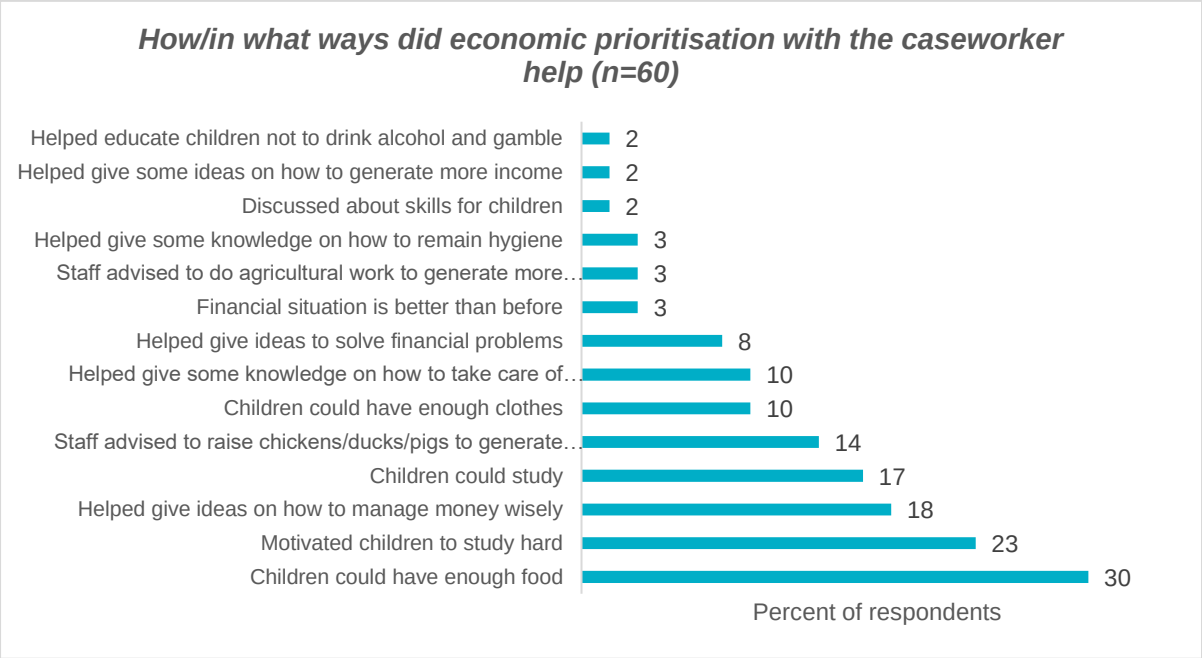
Almost half (44% - 31/70) of the respondents think that the cash transfers are supposed to help their household meet its basic needs for two weeks, yet the cash transfers are actually monthly. Further follow-up and additional training/notifications may be useful to ensure that beneficiaries are budgeting their assistance appropriately, in conjunction with other household income.



87% (61/70) of the respondents reported that their case worker talked to them about what their family/child's needs are, budgeting and how to prioritize household expenditure. 75% (46/61) of them were talked to about this before their first cash transfer and 98% (60/61) of them agreed that these discussions helped them to better meet their family and children's needs. The talks primarily helped

with management of money and methods for parents to take care of their children's food, health and education.





Finally, 93% (65/70) of the respondents reported that they have not shared the cash transfer with anyone outside of their household.

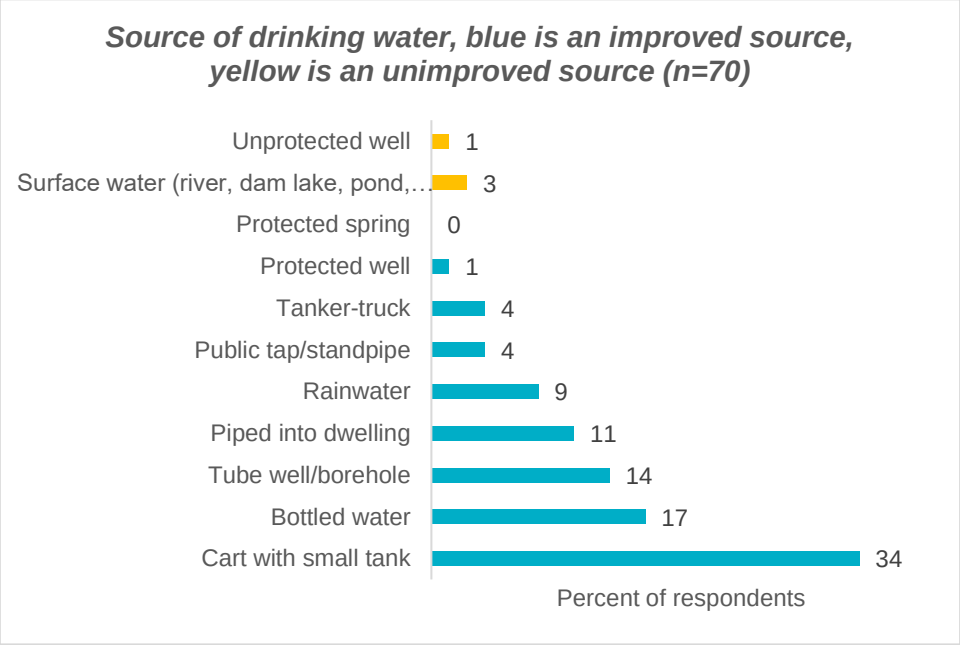
5.7. Percentage of households who report access to safe water for drinking, cooking and cleaning

Key indicator insight:

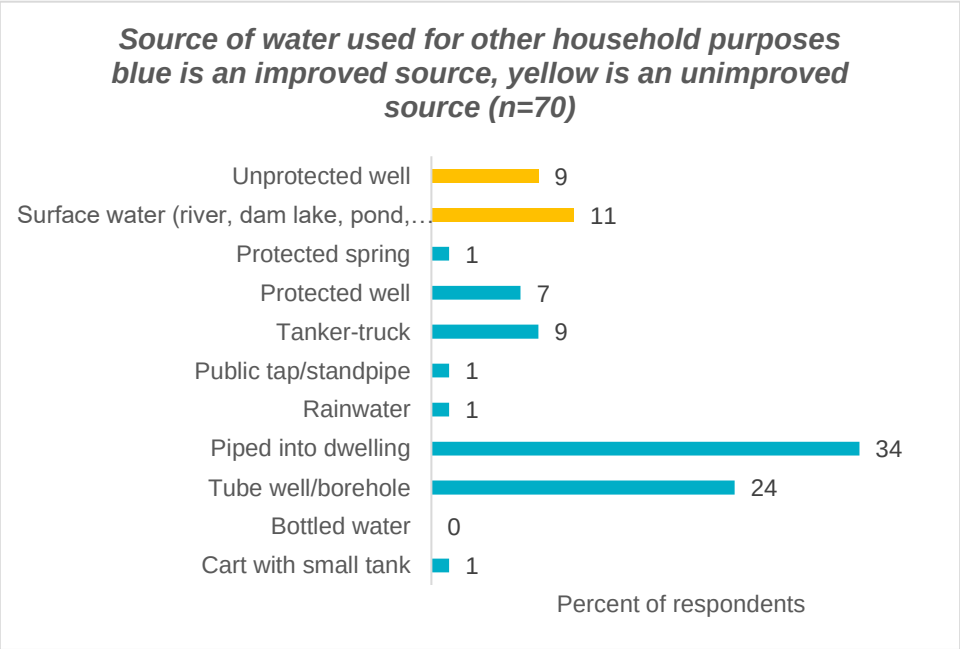
Lack of access to safe drinking water is a major cause of illness, and lack of access to adequate water in the home is also a source of economic disadvantage requiring human resources to fetch and carry water. This indicator looks at access to safe water for ingestion (drinking), cooking and basic personal and domestic hygiene (proxy used in survey is handwashing). Overall, 80% (56/70) of households' report access to safe water for drinking, cooking and cleaning[1].

In total 96% (67/70) of respondents use an 'improved water source' for their drinking water. Drinking water is most often sourced from a 'cart with small tank' as 34% (24/70) of respondents use this method.

[1] For the purposes of this study the indicator measures 'safe water' the same as using an improved water source. The definition of improved water source used in this study is in line with UNICEF/WHO/JMP.



In total respondents use an 'improved water source' for other purposes (80%) less than they do for drinking (96%). Piped water is the most popular source of water for other household purposes with 34% (24/70). 'Cart with small tank', bottled water and rainwater which were popular for drinking water are used by very few respondents for any other tasks. This is likely due to these water sources needing to be purchased, or in the case of collected rainwater, seen as being cleaner and not needing to be treated as much and as such are reserved for drinking purposes.

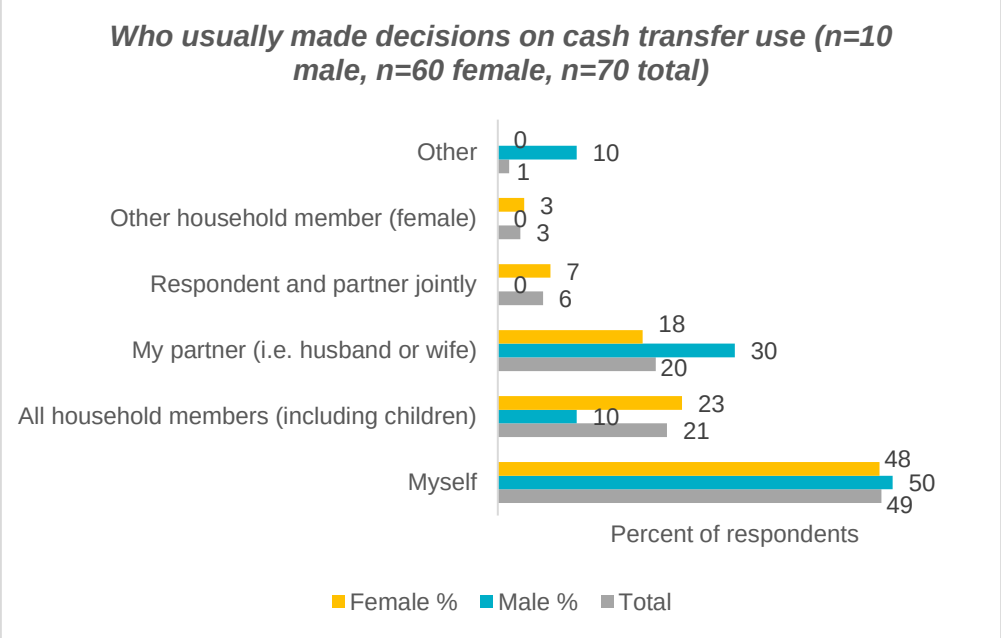


5.8. Percentage of women reporting shared decision-making on cash transfer use

Key indicator insight:

There is evidence that cash transfers increase women's decision-making power, alongside improvements in other empowerment indicators and reduced physical violence due to the cash transfers easing issues that drive these behaviors. Generally, women are reported as being involved in household decision-making if they fully or partially decide on the issue under question. 82% (49/60) of women reported shared decision-making on cash transfer use.

Among both male and female respondents around half make the decision on how to use the money themselves (48% - 29/60 – females, 50% - 5/10 – males). Among female respondents, 23% (14/60) claimed that the decision is made by all household members, including children.



5.9. Percentage of households reporting that the assistance was delivered in a safe manner

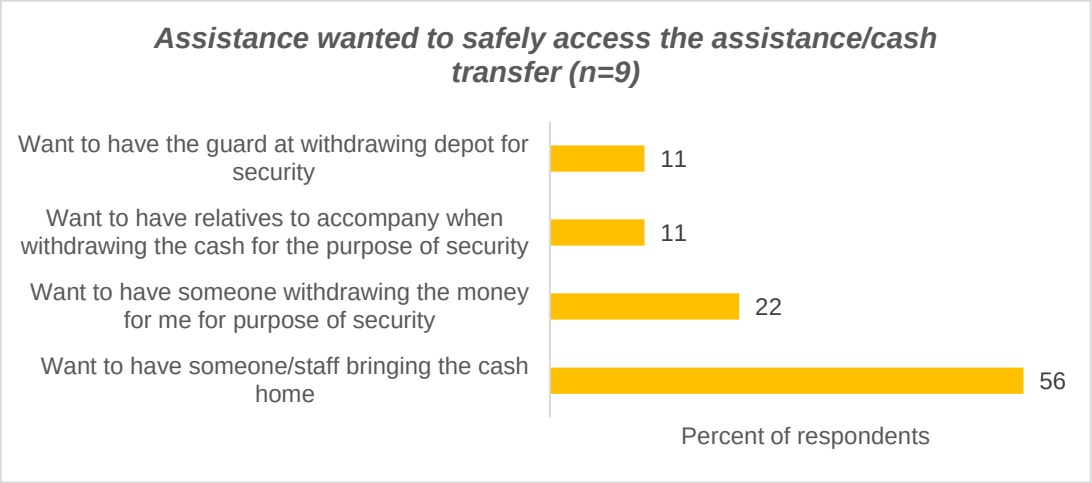
Key indicator insight:

In line with humanitarian and development principles, it is vital that programs are delivered in safe way, which is defined as assistance which prevents and minimizes as much as possible any unintended negative effects of the intervention, which can increase people's vulnerability to both physical and psychosocial risks. 96% (67/70) of respondents reported that they felt safe while receiving the last cash transfer and 4% (3/70) did not feel safe while receiving it, all of whom were female. Although none of the respondents had reported their safety concerns to anyone, two did not feel comfortable explaining why they were uncomfortable and the other mentioned that it was because of safety due to distance/transportation. In all 3 cases the respondent said that they would not like SCI to follow up with them.

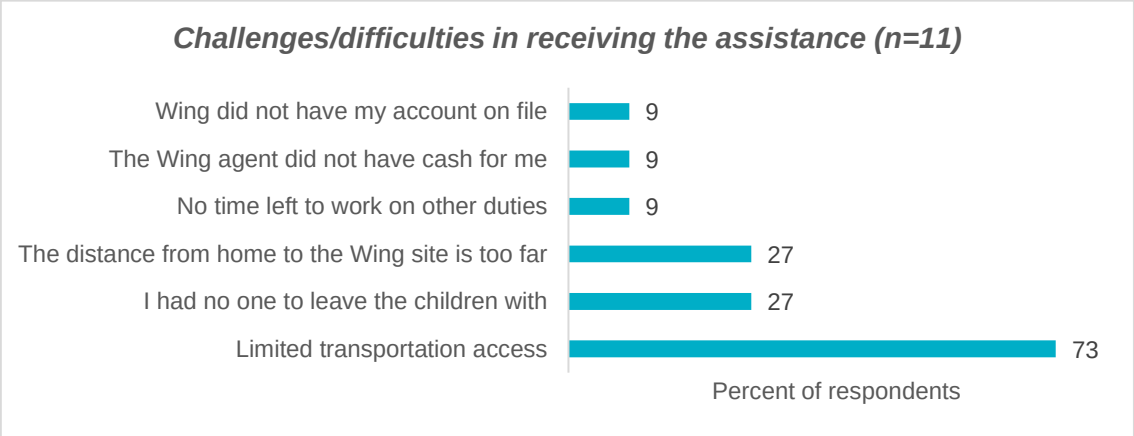
4% (3 respondents) had heard of cases where others had experienced an unsafe situation as a result of receiving the assistance. These 3 respondents were different from the 3 who had safety concerns above. The cases they mentioned were of theft of money received (2 out of 3), fighting/problems within the household receiving the assistance (2 out of 3), and physical assault (1 out of 3).

All respondents were asked if there was anything SCI should be doing to ensure households are safely able to access the assistance provided. 87% (61/70) of respondents believed that there was not and 13% (9/70) said that there was. All respondents who wanted more support for improving safety mentioned support in the collection of the money. 5 of these 9 respondents said that they want SCI to

deliver the money at home, while 3 of remaining 4 wanted a guard or accompaniment when going to collect the money from elsewhere.



Also, 16% of households (11/70) claimed that they had some challenges in receiving the assistance. The single most often mentioned challenge was that the respondent had limited access to transportation 73% (8/11) followed by the distance to the Wing site being too far and not having available childcare, each with 27% (3/11).



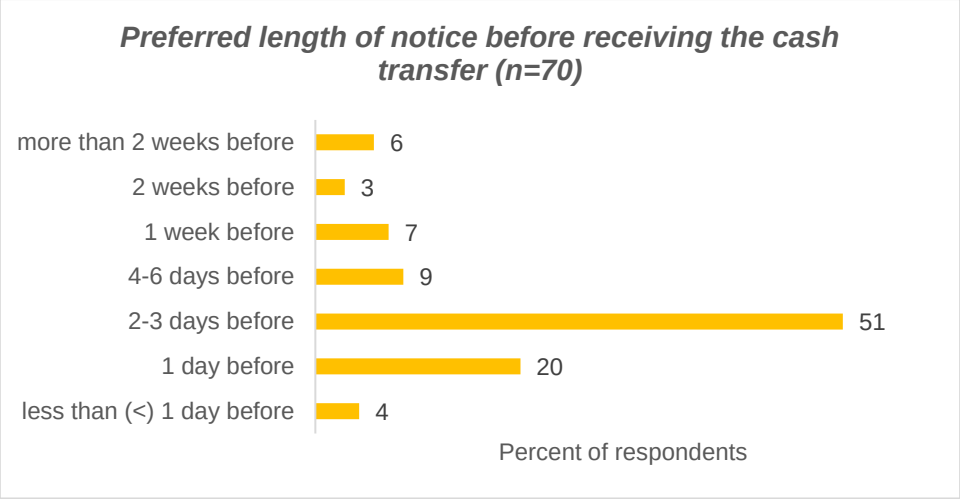
5.10. Percentage of households reporting that they received clear information about the assistance they would receive, including the amount

Key indicator insight:

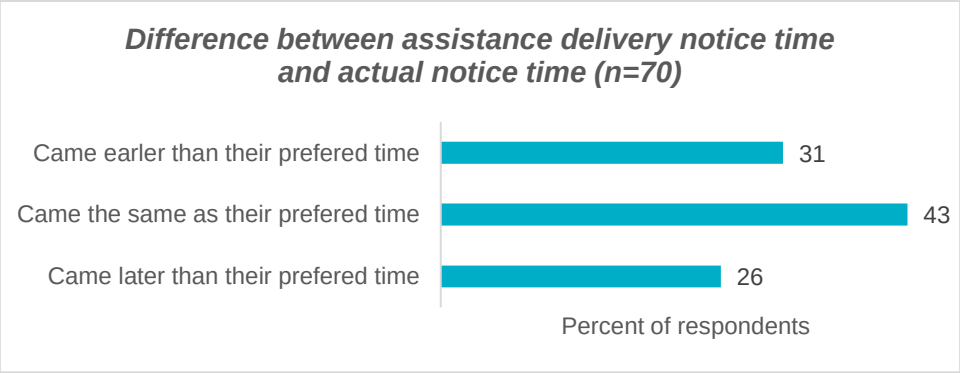
In line with humanitarian and development principles, it is critical to ensure that communities are adequately informed of their entitlements and what to expect from SCI and partners. Overall, 54% (38/70) of households reported that they received clear information about the assistance they would receive, including the amount.

64% (45/70) of households reported understanding why households were selected to receive the cash transfer from SCI/partner, 87% (61/70) received the same amount as they expected and 89% (62/70) of respondents reported that the date of the cash transfer was communicated to them clearly and far enough in advance.

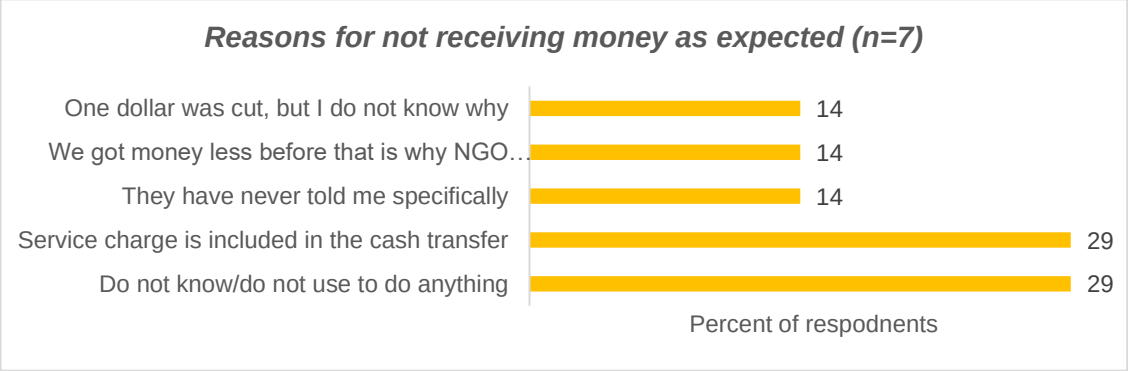
39% (27/70) of the respondents were informed about the date of the cash transfer one day or less than one day before the delivery date with 24% (17/70) being less than one day before. The most preferred notice to be informed about the delivery date of the cash transfer is 2-3 days before (51% - 36/70).



Overall, almost half of the respondents (43% - 30/70) received the notice within the same time frame that they would prefer and just under one third received the notice too early (31% - 22/70) or too late (26% - 18/70) compared to their preference.



In total, 87% (61/70) of the respondents reported receiving the cash transfer amount equal to what they were told they would receive, 3% (2/70) were unsure whether it was correct or not and 10% (7/70) said it was incorrect. Of that 10% who thought their money was not equal to what they were told, 4 of the 7 thought the money was lower than they were told and the remaining 3 thought the money was more. Of these 7, their understanding of why the money was different from what they thought they had been told were almost entirely unique.

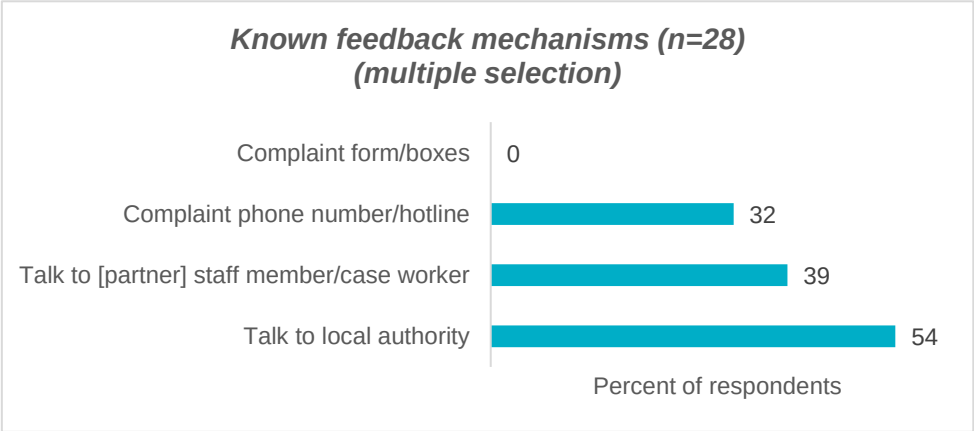


5.11. Percentage of households who can identify at least one available channel for providing complaints or feedback about the assistance

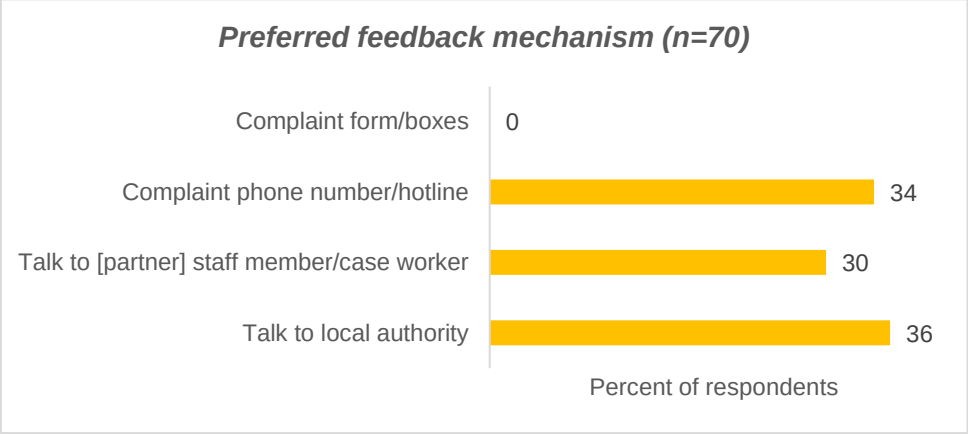
Key indicator insight:

In line with humanitarian and development standards and principles, it is vital to ensure that communities have access to safe and responsive mechanisms to handle feedback, concerns and complaints. If a household reports being aware of a channel for providing complaints or feedback about the assistance, and can then identify at least one available channel, they will be counted towards this indicator. Overall, only 40% (28/70) of households could identify at least one available channel for providing complaints or feedback about the assistance.

Of those who know where to submit a complaint, about half (54% - 15/28) knew of the local authority, with 39% (11/28) identifying partner staff member/case work and about one-third (32% - 9/28) a complaint phone number/hotline. None of the respondents identified complaint form/boxes.



All respondents were presented with the different available options for complaints and asked which they would feel most comfortable using. Most of the options were fairly evenly split with each of the first three options receiving between 30-36%, however, complaint form/boxes received no votes.



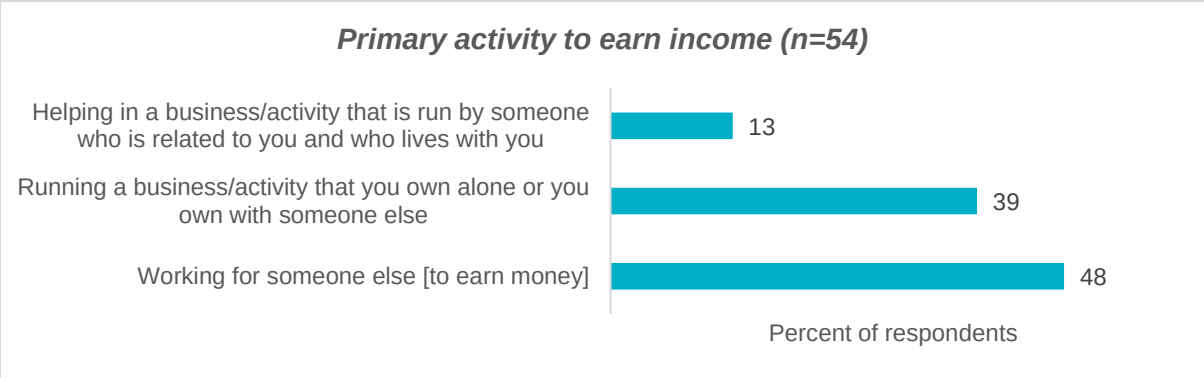
Almost all households, 94% (66/70), reported that they had not or did not know someone who had placed a complaint with a partner/Save the Children. Of the 4 respondents who knew someone who had, 2 replied that the complaints had been resolved, one claimed that it had not, and one did not know.

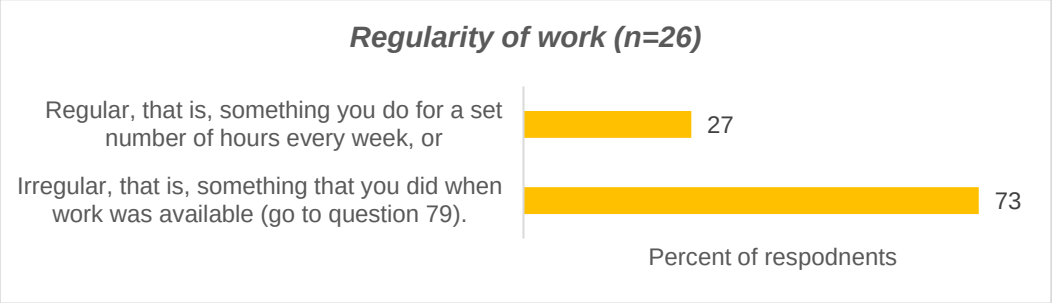
5.12. Average per cent change in earnings following participation in USG-assisted workforce development programs

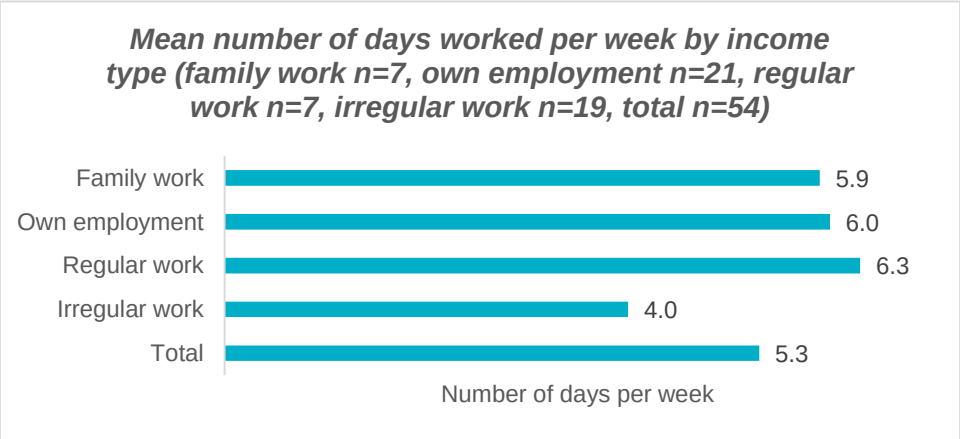
Key indicator insight:

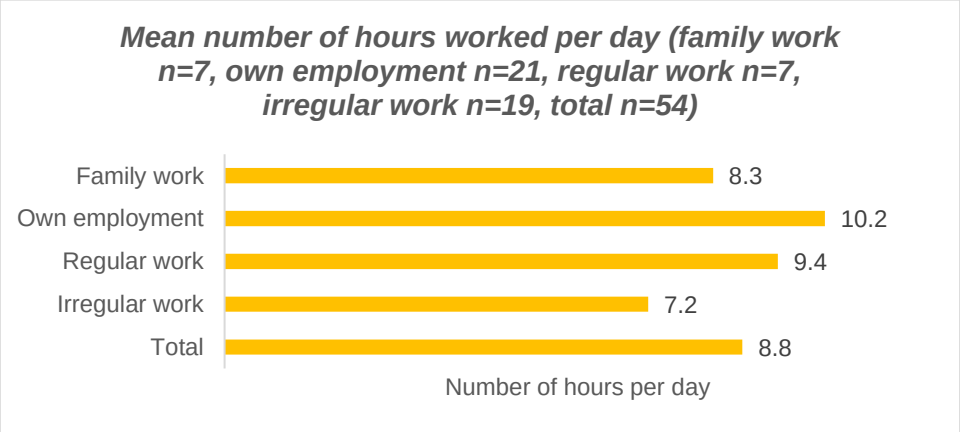
Over time, this indicator will track average percent change in earnings of those who participate in the livelihoods component of this program. The average earnings of those working (54 of 70 respondents) are USD $104.29.

77% of respondents (54/70) did something to earn money. Almost half of them (48% - 26/54) earned their money working for someone else and of those people, most worked irregular hours. Among the different types of work the average number of days worked per week was 5.3, this was highest among those in regular waged jobs at 6.3 and lowest among the irregular workers at 4.0. The respondents on average work 8.8 hours per day, those who have their own employment reported working the most at 10.2 hours per working day and the irregular workers the least at 7.2 hours.

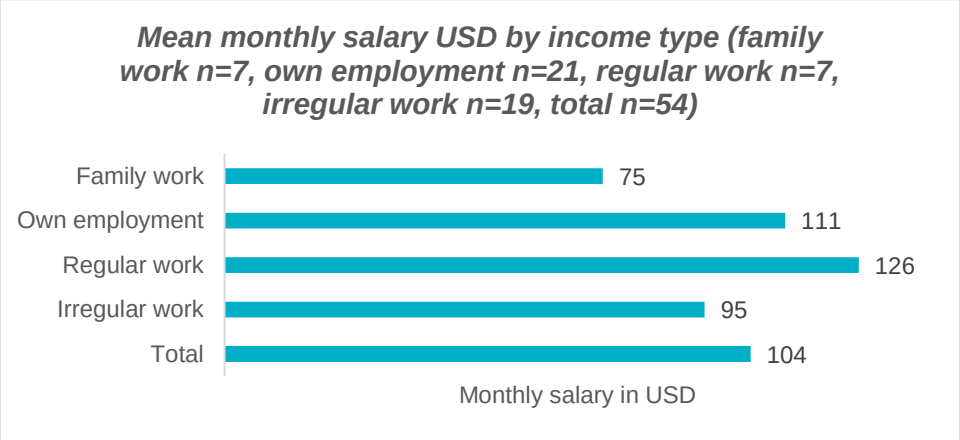








The respondents of the survey were allowed to answer what their average income was over any timeline they defined and in either USD or KHR. Every worker's salary denoted below is calculated as a monthly average and has been converted into USD at a rate of 4,000KHR to 1 USD. For example, if the worker's income was reported in USD for every day they worked, the figure provided would be multiplied by the number of days they work in a week and then that number multiplied by 4.2 weeks in a month to reach the final monthly salary value, in line with USAID guidance on calculation of this indicator. The calculation is not perfect and respondents are somewhat unreliable to accurately remember or to have noted down their earnings. The workers answers are also highly likely to be dependent upon seasonal changes and other external factors (both including and excluding COVID-19). Yet, the results provide an indication of the incomes of various different types of workers and should suffice to remain consistent enough to be measured between the Baseline and Endline surveys.



The income of the workers varied quite substantially. As could perhaps be expected, those who worked for their families were paid the least at an average of $75 USD per month and those who were in regular work were paid the most at $111.14 USD per month with the average salary being $104.29 USD per month.

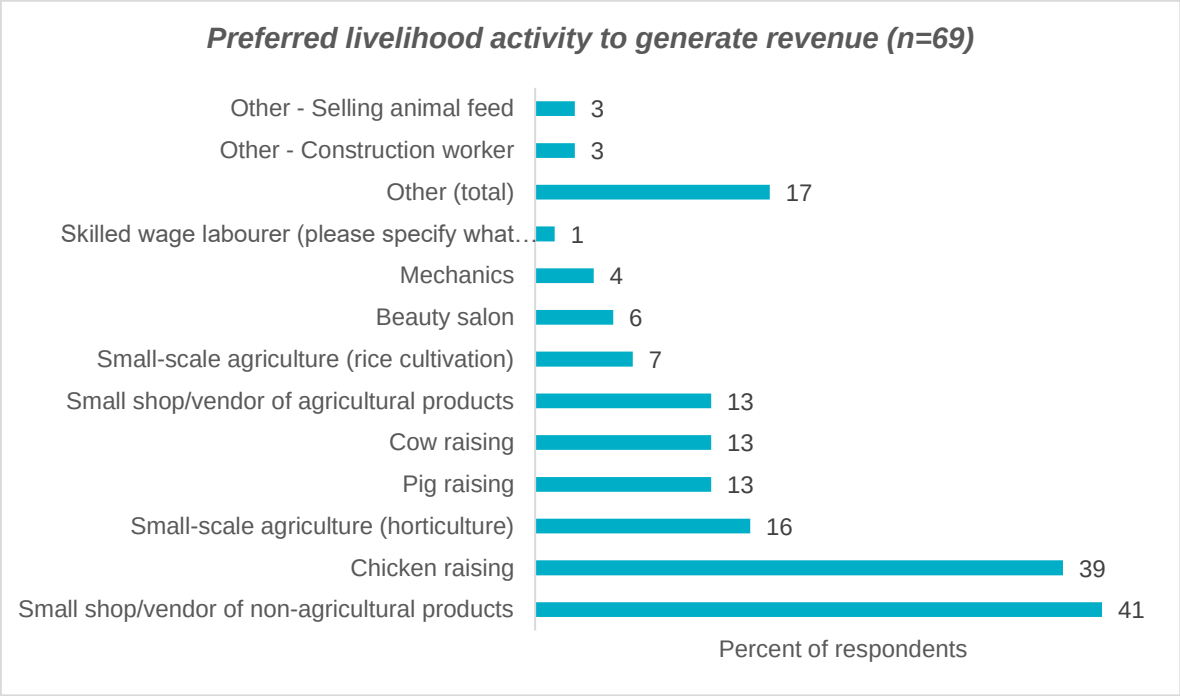
5.13. Percentage of individuals with new employment following participation in USG-assisted workforce development programs

Key indicator insight:

Over time, this indicator will track individuals with new employment following participation in the livelihoods component of this program. Currently, 77% (54/70) of respondents are in paid employment, for the 23% (16/70) who do not earn money, 25% (4/16) of them earn something else in return for their work. The remaining 75% (12/16) did not work for a myriad of reasons. Most prominently (58% - 7/12) because they had to 'take care of the home, children and/or older family members.' For those who are not currently earning money 31% (5/16) are looking for work and 75% (12/16) would be willing to take a job if offered.

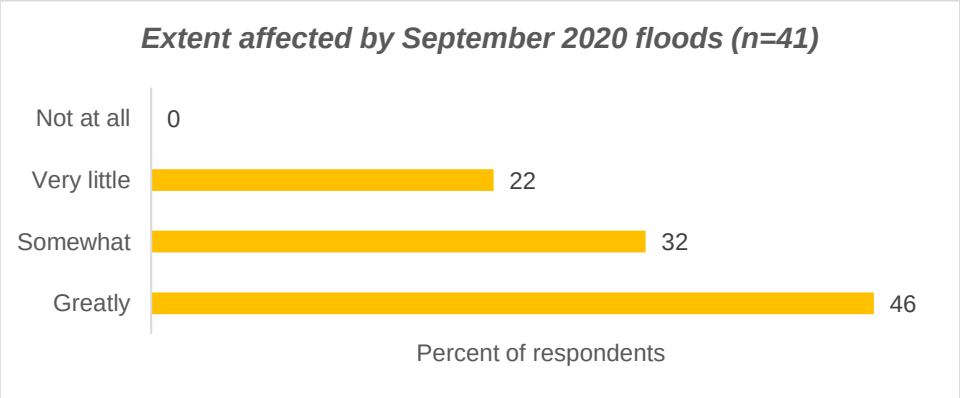
99% (69/70) respondents reported that their household needed support to restore existing/develop new livelihood activities. Of those, 100% (69/69) claimed that they could dedicate more time to restore their existing/develop new livelihood activities. The livelihoods activities which are preferred by the respondents are 'small shop/vendor of non-agricultural products' (42% - 29/69) and 'chicken raising'

(39% - 27/69). Interestingly 'skilled wage laborer' (1% - 1/69) is the least selected occupation of the sample. This fits with the Cambodian cultural ideals of being a business owner.

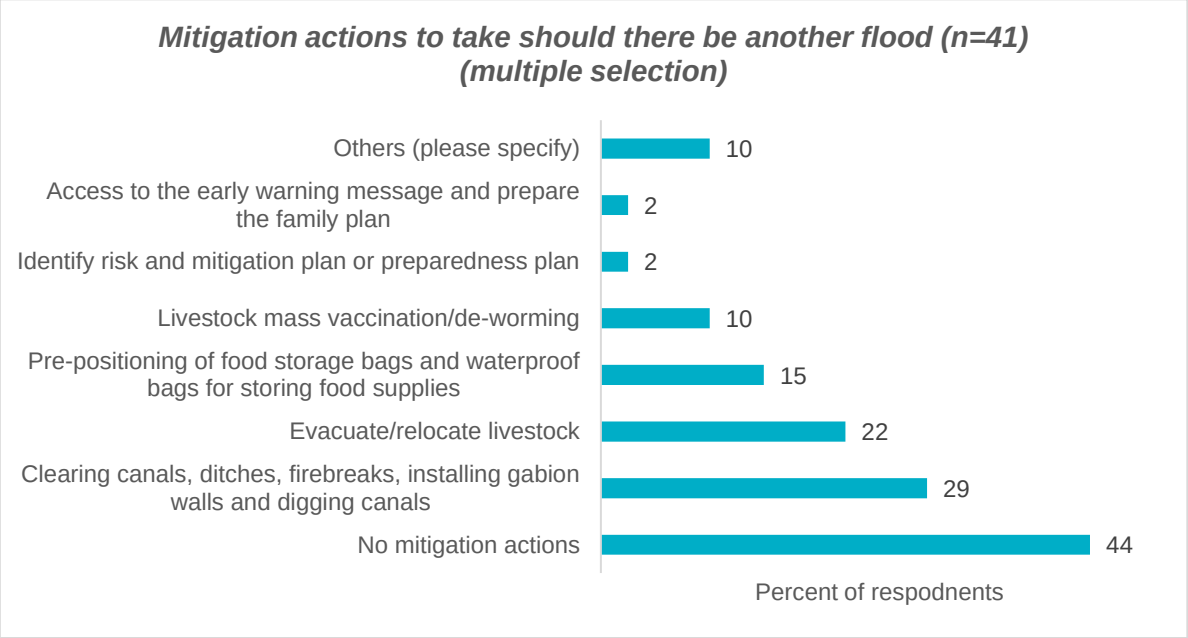


5.14. Other risks

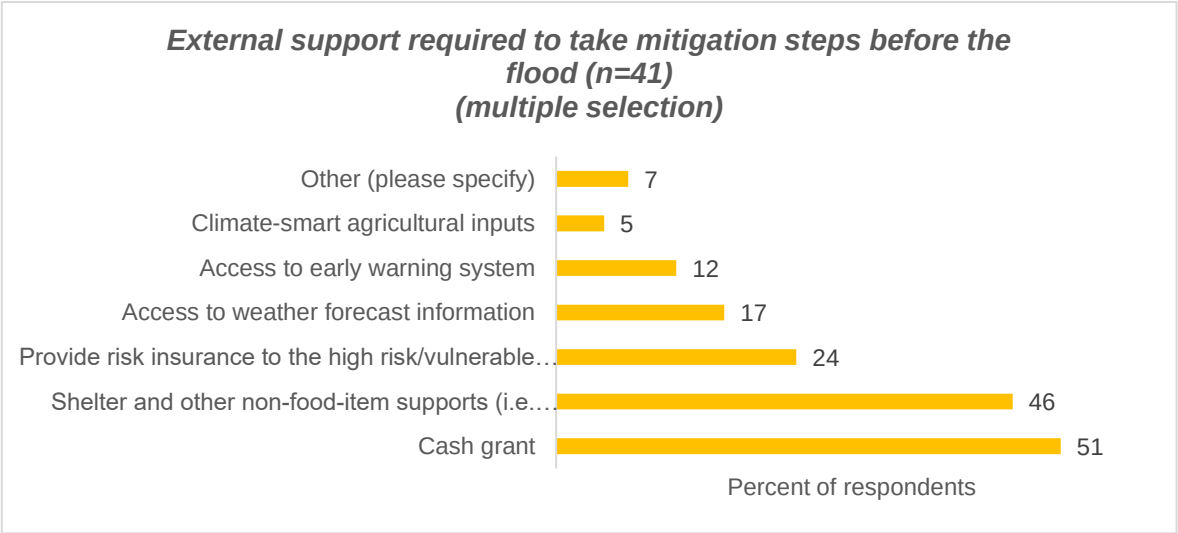
59% (41/70) of the respondents were affected by the floods of September 2020. Of those, many were affected greatly (46% - 19/41).



Although the floods had some impact on all 41 respondents who identified having been affected by them, 44% (18/41) confirm that they would not take any mitigating actions should the floods happen again. Of the 19 respondents who were affected 'greatly' by the floods 58% (11/19) would take no mitigating actions. Among those who would take actions the most popular would be to 'clear canals, ditches, firebreaks, install gabion walls and dig canals' (29% - 12/41) and to 'evacuate/relocate livestock' (22% - 9/41). Interestingly, the least popular mitigation plans were those which allow the respondents to plan for floods for when they eventually happen, rather than reacting to mitigate damage once they are going to occur as 'identify risk and mitigation plan or preparedness plan' (2% - 1/41) and 'access to the early warning message and prepare the family plan' (2% - 1/41) were each least selected.



The primary external support that respondents felt was required to take mitigation steps before the flood would be 'cash grants' (51% - 21/41). Although reported at a lower rate, this is in line with one of the earlier questions in which 96% (67/70) of respondents believed that cash transfers were the most effective form of assistance to help their families. Second to 'cash grants' was shelter and other non-food items at 46% (19/41) of respondents who had previously been through the floods.



6. Recommendations for future implementation

1. **Continue to implement the cash transfers to reduce family separations.**
   The evidence from the baseline suggests that the cash transfers are effective in reducing family separations as migrations have reduced since the cash transfers have started. Furthermore, 96% of respondents suggested that cash was the most effective form of assistance for the family to meet their basic needs.

2. **Improve the communication with recipients on cash delivery.**

Only 54% of respondents reported that they received clear information about the assistance they would receive including the amount. This was largely due to their understanding of why households were selected to receive cash (64%). Improvements could also be made to provide notifications on the cash delivery within the preferred timeline of 2-3 days before it is delivered.

3. **Improve messaging on how long the cash should last and how to budget for their usage.**
   44% of the respondents thought that one cash transfer is supposed to help meet household basic needs for 2 weeks. Although 87% of respondents reported their case worker talking to them about what their family/child's needs are, budgeting and how to prioritize household expenditure, further messaging and training on how to ensure beneficiaries are budgeting assistance appropriately, in conjunction with other household income, in line with their identified needs, would be beneficial.

4. **Improve messaging on channels to file complaints or provide feedback.**
   Only 40% of the respondents could identify a channel to provide complaints or feedback about the assistance. Efforts should be made to increase the recipient's understanding of the feedback channels. Each of the 3 other methods, local authorities, speaking to partner staff/case workers and complaint phone number/hotline had almost equal preference.

5. **Conduct awareness raising on safe migration practices to help families who must travel.**
   Some families remain with some members who have to migrate for their work or education. As such, awareness-raising on how to safely handle this situation should be integrated for the households. Included within this could be knowledge on how the workers can safely migrate, planning for who will care for their children if parents/caregivers must migrate, how the parent/caregiver can keep in touch with the children while they are gone.

6. **Case workers should include discussions about stress, tension and anxiety reduction.**
   More than one third (36%) of households reported that in the past month there had been more stress and tension in the home, and/or household members felt worried or anxious. Methods should be taught to the households for how they can reduce stress, tension and anxiety, especially as COVID-19 restrictions have continued to tighten between the dates of fieldwork and reporting.

7. **Consider the inclusion of food provisions in addition to cash transfers.**
   87% of households identified food as one of their top 3 needs and 97% reported that they spent at least part of their cash transfer on food. If, for example, rice or another staple food could be purchased in bulk and provided along with the cash transfers it would reduce the families need to purchase food at more expensive prices and allow them more money for other necessities.

8. **Focus livelihood development activities on animal raising, agriculture and small shop/vendor.**
   87% of respondents (89% of males and 88% of females) would prefer to focus on animal raising (chicken 39%, cow 13%, pig 13%) agriculture (horticulture 16%, rice cultivation 13%), or small shop/vendor (non-agricultural products 41%, agricultural products 13%) for their livelihood activity to generate more revenue/resources. As such, livelihood development activities within the program should focus on these three broad activities.

7. Annexes

7.1. Evaluation Framework

	Indicator	Monitoring Activities for Collection	Baseline
1	% of households reporting child separation from caregiver (including because of work-related migration) because of lack of money.	Baseline (+PDM); PDM 2; End line	**Before the cash transfer** 21% of households (15/70) reported child separation from caregiver due to lack of money. **Since the cash transfer** 14% of households (10/70) reported child separation from caregiver due to lack of money.
2	% of households that reported having had children go to work during the past 30 days either regularly (on a weekly basis) or frequently (more than once a week), in order to meet basic needs.	Baseline (+PDM); PDM 2; End line	10% (7/70) of households reported having had children go to work during the past 30 days either regularly (on a weekly basis) or frequently (more than once a week), in order to meet basic needs.
3	% of children who were accessing basic education before COVID and are now no longer doing so.	Baseline (+PDM); PDM 2; End line	11% (13/118) of children who were accessing basic education before COVID are no longer doing so.
4	% of households that report feelings of stress and anxiety	Baseline (+PDM); PDM 2; End line	36% (25/70) of households reported that in the past month there has been more stress and tension in the home and/or household members feel worried or anxious and/or there has been some violence in the home.
5	% of households who report being able to meet the basic needs of their households (all/most/some/none) according to their priorities	Baseline (+PDM); PDM 1; PDM 2; End line	4% (3/70) of households reported that they can fulfil all their basic needs, 27% (19/70) can fulfil most of their basic needs while 69% (48/70) can still only fulfil some of them.
6	Average Reduced Coping Strategies Index (rCSI) Score	PDM 2; End line	
7	Prevalence of households with moderate or severe Household Hunger Scale (HHS) score	PDM 2; End line	
8	% of households who report access to safe water for drinking, cooking and cleaning	Baseline (+PDM); PDM 2	80% (56/70) of household's report access to safe water for drinking, cooking and cleaning.
9	% of women reporting shared decision-making on cash transfer use	Baseline (+PDM); PDM 1; PDM 2	82% (49/60) of women reported shared decision-making on cash transfer use.
10	% of households reporting that the assistance was delivered in a safe manner	Baseline (+PDM); PDM 1; PDM 2	96% (67/70) of respondents reported that the assistance was delivered in a safe manner.
11	% of households reporting that they received clear information about the assistance they would receive, including the amount	Baseline (+PDM); PDM 1; PDM 2	54% (38/70) of households reported that they received clear information about the assistance they would receive, including the amount.

12	% of households who can identify at least one available channel for providing complaints or feedback about the assistance	Baseline (+PDM); PDM 1; PDM 2	40% (28/70) of households could identify at least one available channel for providing complaints or feedback about the assistance.
13	Average percent change in earnings following participation in USG-assisted workforce development programs	Baseline (+PDM); End line	The average money earned by those who work is $104.29 per month (54 of 70 respondents)
14	Percent of individuals with new employment following participation in USG-assisted workforce development programs	Baseline (+PDM); End line	77% (54/70) of respondents worked in the previous month to earn money.
15	Percent of individuals with better employment following participation in USG-assisted workforce development programs	End line	
16	Number of individuals in the agriculture system who have applied improved management practices or technologies with USG assistance	End line	

7.2 Research Materials and Tools

Research materials and tools were developed in English by SCI utilizing a number of different existing tools to be able to report against the indicators identified above. Tools utilized in this development included:

- Alliance for Child Protection in Humanitarian Action CVA and Child Protection Tool 2 and Tool 3
- Save the Children International Cash Post-Distribution Monitoring tool
- Save the Children US Emergency Food Security and Livelihoods Post-Distribution Monitoring template
- ECHO Protection Mainstreaming Indicator Questionnaire
- USAID Measuring Employment and Earnings using the Workforce Outcomes Reporting Questionnaire (WORQ)

The tools were submitted to the Save the Children US Ethics Review Committee for approval (SCUS-ERC-FY2021-25). Full materials included in annexes 6.3 Baseline +PDM, 6.4 PDM and 6.5 Last PDM below.

7.3 References

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7.4 Baseline + PDM research tool

FCF|REACT Cash & Livelihoods

Baseline (+PDM) Tool

Introduction and Consent

"Hello. I am [*insert name*]. I work for [*insert name of your company*] and today I am here on behalf of Save the Children to conduct a survey with randomly selected households who are participating in our cash transfer and livelihoods program. The questions will take about 30-45 minutes to complete".

"I would like to ask you some questions about your experience receiving cash transfer support from Save the Children and [*name of partner*]. The questions focus on safety and child protection benefits related to the cash transfer and will help Save the Children and *[name of partner*] improve the program and the safety of those involved as a part of child protection case management."

"Your participation is voluntary. If you agree to participate, you can choose to skip any and/or all questions or choose to end the survey at any time. Whether you participate or not, or your answers, will not affect or change the level or frequency of cash you or anyone else gets in the future."

"If you do not understand a question, please ask me to explain it. Your answers are confidential. That means that I may share details of what you have told me so that we can improve our work, but I will not tell anyone who told me these things. However, if you tell me that you know a child who has been harmed, and continues to be at risk, I would have to tell Save the Children so we could get help for the child, and so we can find ways to prevent that child or other children from being harmed again."

"Do you have any questions for me or concerns you want to discuss with me, before we begin?"

"If you have any concerns you want to share after this interview, or there are any issues with the way we carried out this interview, you may contact the following person [*insert details for company*].

"Do you agree and consent to be interviewed?"

- Yes
- No – if they say no, thank them for their time and terminate the interview.

Client Details

Household ID number: [see HES]

1. Is the respondent the same as for the HES?
   - Yes
   - No

   1a. Respondent Name:

   1b. Respondent Gender:

   1c. Respondent Age:

   1d. Respondent Disability Status:

Cash Transfer Utilization, Information Basic Needs & Decision-Making

2. Do you understand why households were selected to receive the cash transfer from Save the Children/Partner?

   - Yes
   - No

3. What are the 3 priority needs of the household (in order of importance)

   - Food
   - Clothing
   - Shelter
   - Health and medication
   - Water, sanitation and hygiene
   - Education

- Child care
- Repay debts
- Savings
- Livelihoods inputs
- Communications (i.e. phone, internet)
- Psychosocial/mental health services
- Legal/justice services
- Other, specify

4. How much cash in USD/Riel did you receive from Save the Children/Partner in the last distribution? *[Note: cash can be received in either USD or Riel so the currency will also need to be noted with the response]*

5. Was the amount you received equal to the amount you were told you would receive?

- Yes (go to question 7)
- No
- Don't know (go to question 7)

6. If no, was it more or less than you were told you would receive?

- Less
- More

6a. Do you know why there was a difference between what you were told you would receive and what you actually received?

7. Was the date of the cash transfer communicated to you clearly and far enough in advance?

- Yes
- No

8. How long before the cash transfer were you informed about the date of the cash distribution?

- less than (<) 1 day before
- 1 day before
- 2-3 days before
- 4-6 days before
- 1 week before
- 2 weeks before
- more than 2 weeks before

9. How far ahead of time would you ideally like to be notified about the date of the next cash distribution?

- less than (<) 1 day before
- 1 day before
- 2-3 days before
- 4-6 days before
- 1 week before

- 2 weeks before
- more than 2 weeks before

10. For how long do you think one cash transfer is supposed to help your household meet its basic needs?

- One week
- Two weeks
- Three weeks
- Four weeks/one month
- More than four weeks/one month

11. Is your household able to meet all, most, some, or none of its basic needs, as you define them [in the last month]?

- All (go to question 13)
- Most
- Some
- None

12. Why did it not cover the basic needs of the household over the [last month]?

13. Do you think cash assistance is the most effective form of assistance to help you and your family to meet your basic needs?

- Yes (go to question 15)
- No
- Don't know (go to question 15)

14. Why not? And what would be more effective?

15. You mentioned that this Save the Children program gave you ${*amount received*} for the last cash transfer. Can you please tell me which type of your expenses this money went to? *[Multiple selection]*:

- Food
- Clothing
- Shelter
- Health and medication
- Water, sanitation and hygiene
- Education
- Child care
- Repay debts
- Savings
- Livelihoods inputs/invest in income generating activity (if answered, go to 15a)
- Communications (i.e. phone, internet)
- Psychosocial/mental health services
- Legal/justice services
- Other (please specify)

15a. Was the cash you received helpful to restart existing/develop new livelihood activity?

- Yes

- No (go to 15c)

15b. If yes, how was it helpful? (go to question 15d)

15c. If no, why was it not helpful?

15d. How much of the last cash transfer did you invest into your livelihoods?

15e. How much do you think is needed [as a "one-off" amount] to fully restore existing/develop new livelihood activity?

16. Who usually makes decisions about how the cash transfer is used?

- Myself
- My partner (i.e. husband or wife)
- Respondent and partner jointly
- Other household member (female)
- Other household member (male)
- Respondent and other household member jointly
- All adult household members (respondent, partner and others)
- All household members (including children)
- Other (specify)

16a. Did your case worker talk to you about what your family/child's needs are, budgeting and how to prioritize household expenditure?

- Yes
- No (go to question 17)

16b. When did they talk to you about this? [Multiple selection]

- Before the first cash transfer *(prompt: first cash transfer was end of December)*
- After the first cash transfer *(prompt: first cash transfer was end of December)*
- Between second and fourth cash transfers *(prompt: second end of January, fourth will be end of March)*
- Don't remember when

16c. Did these discussions help you to better meet your family and children's needs?

- Yes
- No (go to question 16e)

16d. How/in what ways did it help? (go to question 17)

16e. Why did it not help and how could it be improved?

17. Did you share any of the cash with people not in your household?

- Yes
- No

Access to Safe Water

18. What is the main source of drinking water used by members of your household?

If unclear, probe to identify the place from which members of this household most often collect drinking water (collection point).

Piped water
piped into dwelling .....
piped to yard / plot .....
public tap / standpipe.....
Tube well / borehole.....
Dug well
protected well .....
unprotected well .....
Water from spring
protected spring.....
unprotected spring.....
Rainwater .....
Tanker-truck .....
Cart with small tank .....
Surface water (river, dam, lake, pond, stream, canal, irrigation channel
Bottled water .....

Other (*specify*).....

19. What is the main source of water used by members of your household for other purposes such as cooking and handwashing?

If unclear, probe to identify the place from which members of this household most often collect water for other purposes.

Piped water
piped into dwelling .....
piped to yard / plot .....
public tap / standpipe.....
Tube well / borehole.....
Dug well
protected well .....
unprotected well .....
Water from spring
protected spring.....
unprotected spring.....
Rainwater .....
Tanker-truck .....
Cart with small tank .....
Surface water (river, dam, lake, pond, stream, canal, irrigation channel)
Bottled water .....

Other (*specify*)

Positive & Negative Effects on Household

20. Have the cash transfers had any positive effect on your household or relationships within your household?

- Yes
- No (go to question 22)

21. How has receiving the cash positively affected your household *[multiple selection]*?

- Less physical abuse / violence about expenses
- Less verbal abuse/threats about expenses
- Spouses making decisions together
- Invested in income generating activity
- Postponed child's marriage
- Child does not have to go to work
- Maintained child in school / registered child for school
- Was able to afford health services
- Family members and/or children are healthier
- More food to eat (more quantity)
- Family eating more types of food (more variety)
- Children are eating more often than before
- Adults are eating more often than before
- Other (please specify)

22. Have the cash had any negative effect on your household or relationships within your household?

- Yes
- No (go to question 24)

23. How has receiving the cash negatively affected your household relationships? [NOTE: PLEASE DO NOT READ OPTIONS OUT LOUD] *[multiple selection]*

- Physical abuse / violence about how to spend the cash
- Threat of physical abuse/violence about how to spend the cash
- Verbal abuse about how to spend the cash
- Tensions about who received or how to spend the cash
- Forced withholding of cash
- Husbands or partners demanding sex without consent
- Other (specify)

24. Has this program caused tensions or problems within the community?

- Yes
- No (go to question 26)

25. If you feel comfortable, please tell me what types of problems it has caused.

Safety Receiving Cash Transfer

26. Did you feel safe while receiving the last cash transfer?

This includes safety while going to receive assistance, waiting for assistance and coming back home after assistance

- Yes (go to question 32)
- No

27. If you are comfortable telling me, what type of unsafe situation did you experience in receiving the assistance *[multiple selection]*?

- Theft of money received
- Physical assault
- Verbal assault
- Sexual assault
- Attempted assault
- Lack of respect/consideration
- Fighting/problems in household over assistance
- Safety issues due to distance/transportation
- Safety issues due to water and sanitation facilities (or lack thereof)
- Other (please specify)
- Don't feel comfortable answering

28. Have you already reported this to anyone?

- Yes
- No (go to question 30)

29. Who did you report it to?

30. Would you like a Save the Children staff member to follow-up with you regarding this unsafe situation

- Yes
- No (go to question 32)

31. What is the best and safest way to contact you?

32. Did you hear of anyone else experiencing an unsafe situation or violence as a result of receiving the assistance?

- Yes
- No (go to question 34)

33. If you are comfortable telling me, what type of unsafe situation did you experience in receiving the assistance *[multiple selection]*?

- Theft of money received
- Physical assault
- Verbal assault
- Sexual assault

- Attempted assault
- Lack of respect/consideration
- Fighting/problems in household over assistance
- Safety issues due to distance/transportation
- Safety issues due to water and sanitation facilities (or lack thereof)
- Other (please specify)
- Don't feel comfortable answering

34. Is there anything you think we should be doing to ensure households are safely able to access the assistance/cash transfer?

- Yes
- No (go to question 36)

35. Please detail.

36. Were there any challenges or difficulties you had in receiving the assistance?

- Yes
- No (go to question 38)

37. What were the challenges/difficulties you experienced in receiving the assistance *[multiple selection]*?

- No time left to work on other duties
- Limited transportation access
- The Wing agent did not have cash for me
- Wing did not have my account on file
- I had no one to leave the children with
- Limited childcare arrangements when at the WING site
- Road blockages
- The distance from home to the Wing site is too far
- There are no water and sanitation facilities at the Wing site
- Other (please specify)

Feedback & Complaints Mechanism

38. Are you aware of any complaint or feedback mechanisms where you could submit a concern if you needed to?

- Yes
- No (go to question 40)

39. What feedback mechanisms do you know about *[multiple selection]*?

- Complaint phone number/hotline
- Complaint form/boxes
- Talk to [*partner*] staff member/case worker

- Talk to local authority (i.e. village chief, CCWC, commune chief)

40. Our program has the following complaint mechanisms in place [Enumerator note: Please read the list.] If you had a complaint, which of these mechanisms would you feel most comfortable using?
- Complaint phone number/hotline
- Complaint form/boxes
- Talk to [partner] staff member/case worker
- Talk to local authority (i.e. village chief, CCWC, commune chief)

41. Have you or somebody you know placed a complaint with *Partner*/Save the Children?
- Yes
- No (go to question 43)

42. Was the complaint resolved?

- Yes
- No
- Don't know

Child Protection & Wellbeing

43. Do you have the same number of children living with you now as before the cash transfer started? [*Enumerators should be aware that an answer to this question could be that a child has died and to proceed with the discussion gently].*

- Yes
- No (go to question 46)

44. Did the cash transfer/livelihoods support make it possible for you to keep all your children with you?

- Yes
- No (go to question 49)

45. How did it make this possible? (go to question 49)

46. Do you have more children or less children living with you now?

- More
- Less (go to question 48)

47. If you have more children living with you, why are there more children living with you now? *[multiple selection]* (And then go to question 49)

- Children previously separated were able to return home because of the cash / voucher assistance.
- Child/ren (nor originally part of your household) came to live in your home because you had cash / voucher assistance.
- Child/ren married someone in your household.
- Child/ren came to work in your home.
- Child/ren came to access an education.

- Child/ren came to access health care.
- Child/ren came to be safer.
- Other, please specify.

48. If you now have less children living with you why did the children go? *[multiple selection]*

- Child/ren left because you could not afford to take care of them anymore.
- Child/ren got married.
- Child/ren went to work.
- Child/ren went to access education elsewhere.
- Child/ren left to access health care.
- Child/ren left to be safer.
- Other, please specify.

49. Had any parents/caregivers of children in this household migrated before the cash transfer started (between March - December 2020)?

- Yes
- No (go to question 51)

50. What were the reasons for the migration? *[Can select multiple responses]*

- Household had insufficient income/lack of money
- Employment/work opportunities
- Wanted to work in different province/overseas
- Education opportunities
- Other (specify)

51. Have any parents/caregivers of children in the household migrated since the start of the cash transfer (since December 2020)?

- Yes
- No (go to question 56)

52. What were the reasons for the migration? *[Can select multiple responses]*

- Household had insufficient income/lack of money
- Employment/work opportunities
- Wanted to work in different province/overseas
- Education opportunities
- Other (specify)

53. *[If Yes to either Q.49 or Q.51]* Have any parents/caregivers who migrated before/since the cash transfer started returned to the household?

- Yes
- No (go to question 56)

54. Did they cash transfer make it possible for them to return?

- Yes

- No (go to question 56)

55. How did it make this possible?

56. Over the past 30 days have children in your household participated in work/income generating activities (including begging)?

- Yes
- No (go to question 62)

	Age	Gender	Disability	Engaged in work
Child #1		Male/ Female/ Other	Yes/no	Yes/No
Child #2				
Child #3				
Etc.…				

57. What form of work/income generating where they engaged in?

- Agriculture
- Laborer
- Garment and Textile Industry
- Domestic work
- Family business/trading
- Begging/scavenging
- Other (detail)

58. Did they work regularly in the past 30 days (i.e. recurring shifts/days/times)?

- Yes
- No

59. How frequently did they work in the past 30 days?

- More than once a week
- Less than once a week (go to question 61)

60. Roughly how many hours a week?

- 1-5 hours per week
- 6-10 hours per week
- 11-20 hours per week
- 21-30 hours per week
- 30+ hours per week

61. Why did they start working? *[Can select multiple responses]*

- Household had insufficient income to meet basic needs.
- Schools closed.
- Child wanted to work.
- Other (detail)

62. Before COVID (prior to March 2020), did you have children enrolled and attending school?

	Enrolled/ attending school?	If yes, what grade level
Child #1 (same order as above)	Yes/no	• Tertiary • Upper Secondary (Years 10-12) • Lower Secondary (Years 7-9) • Primary School (Years 1-6) • Early Childhood Education
Child #2	Yes/no	
Child #3	Yes/no	
Etc.…	Yes/no	

63. Are your children currently enrolled in and attending school (whether in-person or remotely)?

	Enrolled/ attending school?	If yes, what grade level	If no, why are they not currently accessing school
Child #1	Yes/no	• Tertiary • Upper Secondary (Years 10-12) • Lower Secondary (Years 7-9) • Primary School (Years 1-6) • Early Childhood Education	• School closed/remote learning not available *(go to Q.64)* • Children finished compulsory schooling (completed Year 9) • Cannot afford to send children to school • Children are going to work • Other (detail)
Child #2	Yes/no		
Child #3	Yes/no		
Etc.…	Yes/no		

64. *[If school closed/remote learning not available selected above]* Do you think your children will return to school once it reopens?

- Yes
- No
- Don't know

65. Have you noticed any of the following changes in your household relationships in the past month (select all that apply)?

- We are enjoying more time together
- There is more stress and tension in the home
- Household members feel worried or anxious
- There has been some violence in the home *(go to question 66 and 67 if this response selected)*
- None of these

66. [If violence selected in Q.65] Has the level of violence in your household changed since you started receiving cash or vouchers?

- Yes – Increased
- Yes – Decreased

- No – levels of violence the same

67. [If violence selected in Q.65] Do you feel comfortable to tell us anything about who is experiencing violence and what you think is causing the violence?

68. Has receiving cash / voucher assistance helped address child protection concerns for the child/ren in your care?

- Yes
- No (go to question 70)
- Don't know (go to question 70)

69. [If yes] How? (go to question 71)

70. [If no/don't know] Why not?

Livelihoods

71. In the previous month, meaning in [month, year] did you do anything to earn money?

- Yes
- No (go to question 88)

72. Now, think what you did to earn most of your income in [month, year]. Was that activity [select one response only]:

- Helping in a business/activity that is run by someone who is related to you and who lives with you [to earn money] (go to question 85)
- Was it running a business/activity that you own alone or you own with someone else, that is, working for yourself [to earn money] (go to question 82)
- Working for someone else [to earn money]

73. Is the work you did for someone else in [month, year]:

- Regular, that is, something you do for a set number of hours every week
- Irregular, that is, something that you did when work was available (go to question 79)

74. When did you start working at this job [record month, year]

75. In a typical week/seven-day period, how many days do you normally work at this job? [record number of days]

76. On a typical day, approximately how many hours per day do you work at this job [record number of hours]

77. How much is your usual take home pay? [record amount, should be net not gross earnings]

78. What period of time does that payment of your wage cover?

- Daily
- Weekly
- Once every two weeks
- Two times a month

- Once a month
- Other (specify)

[go to question 92]

[Irregular]

79. How many days did you work at this job in the last week/seven days [record number of days]

80. When you last worked at this job, how many hours did you work that day? [record number of hours]

81. How much money did you receive as pay for doing this work in the previous month [or month, year]? [record amount]

[go to question 92]

[Own employment]

82. In a typical week, how many days do you do this work/activity? [record number of days]

83. In a typical day, how many hours do you do this work/activity? [record number of hours]

84. How much did you earn from this work/activity in the last week/seven days OR month? [record amount, time period, should be only profit/net earnings, not gross earnings]

[go to question 92]

[Family work/household enterprise]

85. In a typical week/seven-day period, how many days do you usually help out with or work on this business? [record number of days]

86. In a typical day, how many hours do you usually help out with or work on this business? [record number of hours]

87. How much money did you receive as pay for doing this work in the previous month, that is, in [month, year]? [record amount]

[go to question 92]

88. If no, in the previous month, that is, in [month, year], did you do any work for which you got paid with something other than money?

[prompt: household farm, paid with food, clothes, other items]

- Yes (go to question 90)
- No

89. If no, what is the main reason you did not do anything to earn money or receive payment in some other form in the previous month, that is, in [month, year]? [Check one – probe for main reason if multiple]

- On leave voluntarily
- Ill – self or family illness
- Business closed
- Not the right season
- Could not find work
- In school or in training
- Had to take care of home, children and/or older family members
- Other (specify)

90. In the last four weeks, have you looked for a job to help you earn money or receive payment in some other form?

- Yes
- No

91. If you were offered a job, would you be willing to take it?

- Yes
- No

92. Does the household need support to restore existing/develop new livelihood activity?

- Yes
- No (go to question 96)

93. Would you be able to dedicate more time to restore existing/develop new livelihood activity?

- Yes (go to question 95)
- No

94. If no, why not?

95. What livelihood activity would you prefer focusing on to generate more revenue/resources?

- Small-scale agriculture (horticulture)
- Small-scale agriculture (rice cultivation)
- Chicken raising
- Pig raising
- Cow raising
- Small shop/vendor of agricultural products
- Small shop/vendor of non-agricultural products
- Beauty salon
- Mechanics
- Unskilled wage laborer
- Skilled wage laborer (please specify what specific sector of activity)
- Other (please specify)

Other (Disaster Risk Reduction)

96. Have you been affected by the floods that happened in September 2020?

- Yes
- No [go to close]

97. To what extent have you been affected?

- Greatly
- Somewhat
- Very little
- Not at all

98. If the floods were to happen again, what actions would you take to mitigate the impacts on your livelihoods/households (before the flood)?

- Clearing canals, ditches, firebreaks, installing gabion walls and digging canals to redirect flood waters
- Livestock mass vaccination/de-worming
- Identify risk and mitigation plan or preparedness plan including the identification of the safe area.
- Access to the early warning message and prepare the family plan.
- Evacuate/relocate livestock
- Pre-positioning of food storage bags and waterproof bags for storing food supplies, important documents
- Others (please specify)
- No mitigation actions

99. What external support might you require to take mitigation steps (before the flood)?

- Access to weather forecast information
- Access to early-warning system
- Cash grant
- Provide risk insurance to the high risk/vulnerable household
- Shelter and other non-food-item supports (i.e. shelter strengthening kits, materials)
- Climate-smart agricultural inputs
- Other (please specify)
- No support required

Closing

“Thank you for your time today. Your answers will help us understand your experience participating in the program and help improve the support we provide.

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